User Guide

Mental Health and Access to Care Survey (MHACS)

June 2024

Statistics Canada Statistique Canada

Canada

How to obtain more information
Specific inquiries about this product and related statistics or services should be directed to:

Statistics Canada
Client Services
Centre for Population Health Data
E-mail: hd-ds@statcan.gc.ca

Accessing and ordering information
The Mental Health and Access to Care Survey (MHACS) produces three types of microdata files: master files, share files and public use microdata files (PUMF). Master files and Share files were released in September 2023.

For more information, please consult the following web page:
https://www.statcan.gc.ca/eng/microdata/data-centres.

Master files
The master files contain all variables and all records from the collected survey data and administrative data. These files are accessible at a cost at Statistics Canada for internal use and in Statistics Canada's Research Data Centres (RDC), real time remote access (RTRA), and virtual data lab (vDL). Master files are also subject to custom tabulation requests.

Research Data Centre
The RDC Program enables researchers to use the survey data in the master files in a secure environment in several universities across Canada. Researchers must submit research proposals that, once approved, give them access to the RDC.

Real Time Remote Access
The RTRA system is an on-line remote access facility allowing users to run SAS programs, in real-time, against micro-data sets located in a central and secure location. Researchers submit SAS programs to extract results in the form of frequency tables. Confidentiality of the micro data is automated in the RTRA system. Researchers must complete an application form.

Virtual Data Lab
The vDL is a cloud-based microdata access centre that allows approved researchers to remotely access data housed by Statistics Canada using a secure IT connection and a protected Cloud environment.

Custom tabulations
Another way to access the master files is to offer all users the option of having staff in the Centre for Population Health Data (CPHD) Client Services prepare custom tabulations. This cost-recovery product allows users who do not possess knowledge of tabulation software products to get custom results. The results are screened for confidentiality and reliability concerns before release. For more information, please contact the Client Services at hd-ds@statcan.gc.ca.

Share files
The share files contain all records of the MHACS respondents who agreed to share their data with Statistics Canada's partners. The share file is released only to these partner organizations. Personal identifiers are removed from the share files to respect respondent confidentiality. A Microdata Licence Agreement is signed before releasing the file(s). This requires the name of the person who will be responsible for the data file and their contact information. Users of these files must first certify that they will not disclose, at any time, any information that might identify a survey respondent.

Public use microdata files

The public use microdata files (PUMF) are developed from the master files using a technique that balances the need to ensure respondent confidentiality with the need to produce the most useful data possible. The PUMF must meet stringent security and confidentiality standards required by the *Statistics Act* before they are released for public access. To ensure that these standards have been achieved, each PUMF goes through a formal review and approval process by an executive committee of Statistics Canada. Variables most likely to lead to identification of an individual are deleted from the data file or are collapsed to broader categories.

Table of Contents

1.0 Introduction ... 7

2.0 Survey Content ... 8

2.1 Module Selection ... 8
2.2 Qualitative Testing ... 10

3.0 Survey Design ... 11

3.1 Target and Observed Population ... 11
3.2 Survey Frame ... 11
3.3 Sample Size and Allocation ... 11
3.4 Selection of Interviewees ... 12

4.0 Data collection ... 12

5.0 Data processing ... 12

5.1 Editing ... 12
5.2 Coding open-ended questions ... 13
5.3 Creation of derived variables ... 13
5.4 Linked income variables ... 13
5.5 Weighting ... 13
5.6 Imputation ... 14
5.6.1 Income ... 14
5.6.2 Postal Code ... 14
5.6.3 Gender ... 14
5.7 Disclosure control ... 14

6.0 Weighting ... 15

6.1 Overview ... 15
6.2 Initial weight (wgtT0) ... 15
6.3 Scope-adjusted weight (wgtT5) ... 16
6.4 Nonresponse adjusted weight (wgtT6) ... 16
6.5 Winsorization (wgtT7) ... 16
6.6 Post-stratification (wgtT8) ... 16
6.7 Share Weight ... 17
6.8 Bootstrap Weights ... 17

7.0 Data Quality ... 18

7.1 Survey errors ... 18
7.2 Non-sampling errors ... 18
7.2.1 Non-response ... 18
7.2.2 Coverage errors ... 19
7.2.3 Measurement errors ... 19
7.2.4 Processing errors ... 20
7.3 Sampling errors ... 20
7.4 Bias Evaluation ... 21

8.0 Guidelines for tabulation, analysis and release ... 22

8.1 Rounding guidelines ... 22
8.2 Sample weighting guidelines for tabulation ... 22
8.3 Guidelines for statistical analysis ... 23
8.4 Release guidelines for quality ... 23
8.5 Guidelines for Statistical Analysis, Variance Estimation and Constructing Confidence intervals 26

Appendix A: Comparison of 2012 CCHS - Mental Health, 2002 CCHS - Mental Health and Well-Being, and 2022 Mental Health and Access to Care Survey ... 29

Appendix B: WHO-CIDI Mental and Substance Disorders – Primary variables for analysis and comparable 2022, 2012, 2002 variables 124

1.0 Introduction

The purpose of the Mental Health and Access to Care Survey (MHACS) was to collect information about the mental health status of Canadians, as well as their access to and need for services and supports, whether formal or informal. This survey also intended to assess the impact of the COVID-19 pandemic on population health as well as evaluate changes in patterns of mental health, service use and functioning in the last ten years.

The Mental Health and Access to Care Survey (MHACS) is a voluntary survey that covers the population 15 years of age and over as of March 1, 2022 living in the ten Canadian provinces. Excluded from the survey's coverage are: persons living on reserves and other Aboriginal settlements, full-time members of the Canadian Forces, and persons living in collective dwellings, such as institutional residences. The sample was selected from the population that completed the long questionnaire of the 2021 Census.

The 2022 MHACS was administered by computer-assisted telephone interviews (CATI) due to the COVID-19 pandemic. Data were collected from March 17 to July 31, 2022.

A survey with many of the same questions, called the Canadian Community Health Survey – Mental Health (CCHS-MH), was conducted in 2012. This means that many of the same indicators can be derived for MHACS and CCHS-MH to assess changes in population health that occurring during the intervening decade. Some of the same indicators are also available on the Canadian Community Health Survey: Mental Health and Wellbeing that was conducted in 2002.

There are important differences in the methodologies used for the MHACS and the CCHS-MH that need to be taken into consideration in order to appropriately compare the results of these two surveys. This user guide describes methodology used for the MHACS and is intended to provide the information needed to facilitate analysis using the microdata files.

Any question about the data set or its use should be directed to:

Statistics Canada
Client Services
Centre for Population Health Data
E-mail: hd-ds@statcan.gc.ca

2.0 Survey Content

This section provides a general discussion of the survey content development and gives a summary of the final content selected for inclusion in the MHACS.

A copy of the final questionnaire and associated specs used for data collection can be accessed through the RDC.

Content was selected to address the following survey objectives:

- To assess the mental health status of Canadians on both illness and positive mental health continuums through selected mental and substance disorders, mental health problems, and well-being;
- To assess the impact of the COVID-19 pandemic on population health by evaluating changes in patterns of mental health, service use and functioning since the 2012 Canadian Community Health Survey.
- To assess timely, adequate, and appropriate access to and utilization of formal and informal mental health services and supports as well as perceived needs;
- To assess functioning, ability and disability in relation to mental health and illness; and
- To examine links between mental health and social, demographic, geographic, and economic variables or characteristics (covariates) including health behaviours, health conditions, and childhood experiences.

2.1 Module Selection

The content for the MHACS was based on the content used for the CCHS-Mental Health from 2012. Questions were added or modified to address a data gap that had been identified and/or to meet the current Statistics Canada standards for harmonized content across surveys.

Thirty-one of 39 module overlap with the 2012 CCHS-Mental Health.

- 24 modules from the 2012 survey are repeated with no changes or only minor changes.
- 7 modules from the 2012 survey are repeated with moderate/major changes.
- 8 modules are new in 2022.

A detailed comparison information between the 2002, 2012, and 2022 surveys is available in *Appendix A – Comparison of 2012 CCHS - Mental Health, 2002 CCHS - Mental Health and Well-Being, and 2022 Mental Health and Access to Care Survey* and *Appendix B -- WHO-CIDI Mental and Substance Disorders – Primary variables for analysis and comparable 2022, 2012, 2002 variables*.

Table 1 Provides an overview of the concepts covered on the MHACS.

Table 1. Overview of the Mental Health and Access to Care Survey.

Primary Use	Module(s)	Description	Source / Reference(s)
To estimate prevalence of selected mental disorders based on DSM-IV criteria	Screening Section (SCR) Depression (DEP) Mania (MIA) Generalized Anxiety Disorder (GAD) Social Phobia (SOP) Alcohol Use, Abuse, and Dependence (AUD) Substance Use, Abuse, and Dependence (SUD)	See documentation for Derived Variables for more information about scoring algorithms.	World Health Organization Composite International Diagnostic Interview (CIDI)
To assess diagnosis of chronic conditions (including some mental health conditions)	Chronic conditions (CCC)	This group of questions deals with long-term health conditions, diagnosed by a health professional, that have lasted or are expected to last at least 6 months.	Previously collected on selected years of the Canadian Community Health Survey.
To assess indicators of general health (including mental health)	General Health (GEN)	General health indicators include: self-perceived health, self-perceived mental health, satisfaction with life, self-perceived stress and sense of belonging to local community.	These indicators are part of Statistics Canada's Quality of Life Framework. They are collected on several surveys, including annually on the Canadian Community Health Survey.
To assess indicators related to suicide	Suicide sub block (SUI)	Concepts assessed include suicidal ideation, planning, and attempts	Previously collected on selected years of the Canadian Community Health Survey.
To assess functioning, ability and disability in relation to mental health and illness	Distress (DIS) Pain and discomfort (HUP) WHO Disability Assessment Schedule (DAS)	See documentation for Derived Variables for more information.	DIS: Kessler Psychological Distress Scale HUP: Pain attribute from the Heath Utility Index – 3. DAS: WHO Disability Assessment Schedule
To collect data on mental health care needs and services used	Mental Health Services (SR1) Medication Use (MED) Perceived Need for Care (PNC)	See documentation for Derived Variables for more information.	In addition to the 2012 CCHS-MH, questions from these modules have been previously collected on selected years of the Canadian Community Health Survey.

To collect data on factors associated with mental health and access to care (including health behaviours, childhood experiences and social wellbeing measures)	Sources of Stress (STS) Social Provisions Scale (SPS) Negative Social Interactions (NSI) Spirituality (SPI) Childhood Experiences (CEX) Work Stress (WST) Mental Health Experiences (MHE) Family Mental Health Impact (FMI) Smoking (SMK) Physical Activity (PHS) Impacts of COVID-19 (IM)	See documentation for Derived Variables for more information.	Many of these modules have been used on selected years of the Canadian Community Health Survey.
To assess demographic characteristics	Age (AN3) Sex and Gender (GDR) Socio-demographic characteristics (SDC) Immigration minimum block (IMM) Indigenous identity (ABM) Population group (PGM) module (PG) Education (EDU) Labour market activities (LMAM)	These modules follow the current Statistics Canada standards for harmonized content across surveys. See documentation for Derived Variables for more information.	
Included to collect administrative information including permission for data linkage and data sharing.	Administration information (ADM) Health number validation (HN) Contact information (CON)		

2.2 Qualitative Testing

Questionnaire review of new sections was conducted in English and French through Statistics Canada's Questionnaire Design Resource Centre (QDRC). Since the vast majority of content was copied from the 2012 CCHS Mental Health with minimal changes, the survey did not require in-depth qualitative testing. Please refer to the User Guide for the 2012 CCHS Mental Health for a description of the qualitative testing process that was conducted in 2010.

3.0 Survey Design

3.1 Target and Observed Population

The MHACS covers the population 15 years of age and over as of March 1, 2022, living in the ten provinces of Canada. Excluded from the survey's coverage are persons living on reserves and other Aboriginal settlements, full-time members of the Canadian Forces, and persons living in collective dwellings, such as institutional residences. Altogether, these exclusions represent less than 2.5% of the target population.

3.2 Survey Frame

The MHACS frame is the set of respondents to the 2021 long-form Census 15 years of age and over as of March 1, 2022, living off-reserve in the ten provinces of Canada. The observed population does not include individuals who immigrated to Canada after the census reference date on May 11, 2021.

3.3 Sample Size and Allocation

The MHACS has a stratified simple random sample and cross-sectional design.

The MHACS sample of 40,000 units is allocated with the goal of producing estimates of roughly equal precision within each of the eight strata combining binary gender[1] and age group (15-24, 25-44, 45-64, and 65+). The survey was designed to oversample the people identifying with of the four largest population groups in Canada designated as visible minorities (South Asian, Black, Chinese, Filipino), to support publication of corresponding estimates at the national level. Within each age-gender grouping, the sample is allocated to the five population group strata (‘South Asian’, ‘Black’, ‘Chinese’, ‘Filipino’ and a catch-all stratum ‘Other’ containing the rest of the target population) using a 0.5 power allocation based on the relative size of each of these five population groups.

The allocation of the sample to strata is summarized in Table 2.

Table 2. Sample size by age group, gender and population group.

Age Group	Gender	Filipino	Black	Chinese	South Asian	Other	Total
15 - 24 years old	Man	579	735	691	968	2,729	
	Woman	536	705	661	855	2,539	
	Total						**10,998**
25 – 44 years old	Man	502	648	665	947	2,678	
	Woman	516	621	649	839	2,442	
	Total						**10,507**
45 – 64 years old	Man	429	512	615	671	2,652	
	Woman	467	473	616	594	2,429	
	Total						**9,456**
65+ years old	Man	276	363	584	571	2,725	
	Woman	343	382	580	536	2,678	
	Total						**9,039**
CANADA		**3,649**	**4,438**	**5,060**	**5,981**	**20,872**	**40,000**

[1] A binary gender variable was derived from Census responses to classify each person as either a man or woman strictly for MHACS allocation purposes.

3.4 Selection of Interviewees

The MHACS sample has a one-stage design[2]. A simple random sample of persons is selected independently within each sampling stratum according to the allocation shown in Table 2.

4.0 Data collection

MHACS was conducted using Internet-Based Electronic Questionnaires (EQ), which were tested in both official languages throughout the development. Over the collection period, a total of 9,861 valid interviews were conducted using an interviewer-led method conducted by telephone (iEQ). The 2012 CCHS Mental Health used a type of computer-assisted interviewing (CAI) called computer assisted personal interviewing (CAPI). To maintain consistency with the 2012 survey, interviewers received extensive training on the survey content and MHACS respondents were not permitted to self-complete the interview.

Responses to survey questions were captured directly by the interviewer at the time of the telephone interview using a computerized questionnaire which mimics the electronic questionnaire application. The computerized questionnaire reduces processing time and costs associated with data entry transcription errors and data transmission. The responses provided by respondents were secure through industry standard encryption protocols, firewalls, and encryption layers.

The data collection was conducted between March 17 to July 31 of 2022. Responding to this survey was voluntary.

5.0 Data processing

Processing transforms survey responses obtained during collection into a form that is suitable for tabulation and data analysis. It includes all data handling activities – automated and manual – after collection and prior to estimation.

5.1 Editing

Editing can occur at several points throughout the survey process and ranges from simple preliminary checks performed by the electronic questionnaire application to more complex automated verifications performed at the processing stage after the data have been collected. In general, edit rules are determined by what is logically or validly possible, based upon:

- expert knowledge of the subject matter;
- other related surveys or data;
- structure of the questionnaire and its questions; and
- statistical theory.

There are three main categories of edits: validity, consistency and distribution edits. Validity edits verify the syntax of responses and include such things as checking that the data lie within an

[2] For weighting and variance estimation purposes, the overall MHACS sample design is considered as having two phases, the first being the sample of Canadians selected to receive the 2021 Census of Population long-form and the second corresponding to the MHACS sample.

allowed range of values. For example, a range edit might be placed on the reported age of a respondent to ensure that it lies between 0 and 121 years.

Consistency edits verify that relationships between questions are respected. Consistency edits can be based on logical, legal, accounting or structural relationships between questions or parts of a question. The relationship between date of birth and marital status is one example where an edit might be: “a person less than 15 years of age cannot have any marital status other than single - never married.”

Distribution edits are performed by looking at data across questionnaires. These edits attempt to identify records that are outliers with respect to the distribution of the data. Distribution edits are sometimes referred to as statistical edits or outlier detection.

5.2 Coding open-ended questions

A few data items on the questionnaire were recorded in an open-ended format. The standard method of coding open-ended questions is to review the responses to determine if the response fits into an existing category. If the response was not fitting an existing category, the response was left in the generic other category.

5.3 Creation of derived variables

In order to facilitate data analysis, a number of data items on the microdata file were derived by combining items on the questionnaire. For example, answer categories of one, two or more variables may be combined to create a new variable. The data dictionary identifies which variables were derived and the nature of their derivation. Supplemental documentation is available detailing each of the derived variables.

5.4 Linked income variables

The MHACS does not ask income questions directly to the respondents. Rather, the income variables are obtained from administrative data sources to reduce response burden and to improve data quality.

The MHACS sample is drawn from the 2021 Census of Population long form respondents and the Census database already contains income variables of interest. These income variables were added to the Census database through linkage to tax data and had already undergone editing and imputation processes.

The income data for the MHACS are obtained through linkage to the 2021 Census and are appended to the MHACS master file for every respondent who did not object to having their responses linked.

For the share file, since the data linkage consent was not asked to all members of the household, the total 2020 household income is modelled using a set of variables that are correlated with household income.

5.5 Weighting

The principle behind estimation for a survey with a probability sample such as MHACS is that each person in the sample represents, besides himself or herself, several other persons not in the sample. For example, in a 2% simple random sample of the population, each person in the sample represents 50 persons in the population. In the terminology used here, it can be said that each person has a weight of 50.

The weighting phase is a step that calculates, for each person, their associated sampling weight. This weight appears on the microdata files and must be used to derive meaningful estimates from the survey. For example, if the number of individuals who have asthma is to be estimated, it is done by selecting the records referring to those individuals in the sample having that characteristic and summing the weights corresponding to those records.

Details of the method used to calculate sample weights are presented in Chapter 6 “Weighting”.

5.6 Imputation

Imputation is a process used to determine and assign replacement values to resolve problems of missing, invalid or inconsistent data. This is done by changing some of the responses and all of the missing values on the record being edited to ensure that a plausible, internally consistent record is created.

5.6.1 Income

Income variables were imputed for the share file because the data linkage consent was not asked to all members of the household. The total 2020 household income was modelled using a set of variables that are correlated with household income.

5.6.2 Postal Code

A respondent’s postal code was used, together with the Postal Code Conversion File, to derive the geographical variables that are available on the MHACS data files. It is therefore important that all respondents have a valid postal code.

If a respondent’s postal code was missing or invalid, it was usually imputed through a donor imputation process, although other imputation methods were sometimes used. The donor was chosen from the same geographical area as the respondent with the missing or invalid postal code with as much precision as possible.

For the MHACS, 4.5% of respondents had their postal codes imputed.

5.6.3 Gender

To reduce the risk of residual disclosure, a two-category gender variable was created.

Gender refers to an individual's personal and social identity as a man, woman or non-binary person (a person who is not exclusively a man or a woman). Given that the non-binary population is small, data aggregation to a two-category gender variable is sometimes necessary to protect the confidentiality of responses provided. In these cases, individuals in the category "non-binary persons" are distributed into the other two gender categories and are denoted by the "+" symbol. "Men+" includes men (and/or boys), as well as some non-binary persons. "Women+" includes women (and/or girls), as well as some non-binary persons.

5.7 Disclosure control

Statistics Canada is prohibited by law from releasing any information it collects that could identify any person, business, or organization, unless consent has been given by the respondent or as permitted by the Statistics Act. Various confidentiality rules are applied to all data that are

released or published to prevent the publication or disclosure of any information deemed confidential. If necessary, data are suppressed to prevent direct or residual disclosure of identifiable data.

6.0 Weighting

For estimates produced from survey data to be representative of the covered population, and not just representative of the sample itself, users must incorporate the survey weights in their calculations. A survey weight is given to each respondent included in the final sample. This weight corresponds to the number of persons in the entire population that are represented by the respondent.

As stated in section 3.2, the MHACS is a single-frame survey based on the 2021 Census of Population long form respondents. The weights of units selected for the survey go through several weight adjustment steps before arriving at the final master and share weights.

6.1 Overview

Many adjustments to unit weights are made throughout the weighting process. These adjustments are enumerated from T0 to T8 and described in Diagram A. The following sections present an overview of the initialization of the weights (T0) as well as adjustments T5-T8, the most important adjustments that are part of the weighting strategy.

Diagram A - Weighting Strategy Overview

Survey Frame

T0 – Initial weight (wgtT0)
T1 – Linkage-scope-adjusted weight (wgtT1)
T2 – Collection-adjusted weight (wgtT2)
T3 – Overlap-adjusted weight (wgtT3)
T4 – Stabilized weight (wgtT4)
T5 – Scope-adjusted weight (wgtT5)
T6 – Nonresponse adjusted weight (wgtT6)
T7 – Winsorization (wgtT7)
T8 – Calibration (wgtT8)

Final Survey Weight

6.2 Initial weight (wgtT0)

The initial design weight is defined as the inverse of the probability of selection. The sample was selected from the frame based on the 2021 Census respondents. Since the 2021 Census long-form itself is a sample[3], persons on the MHACS frame already come with their own weight.

The initial weight of a unit for MHACS (wgtT0) is therefore a product of the following two quantities: 2021 Census weights (wgtCensus2021), and MHACS person-selection adjustment (adjMHACS = inverse of the probability of person-selection, or the number of persons on the frame in the unit's stratum divided by the number of sampled persons in that stratum). The initial unit weight, wgtT0, is given by the following formula:

$$wgtT0 = wgtCensus2021 * adjMHACS$$

[3] For Census 2021, a sample (approximately ¼) was taken in mail-out, list-leave and seasonal areas, but in canvasser and reserve areas, the sampling fraction was equal to 1 and all dwellings received the long-form questionnaire.

6.3 Scope-adjusted weight (wgtT5)

Among all sampled persons, a certain proportion are identified during collection as being out-of-scope or as being ineligible for the survey. People who are full-time members of the Canadian Armed Forces, people living on reserve, children younger than 15 years old, as well as persons who have moved outside of the ten provinces or who are deceased are examples of out-of-scope cases for the MHACS. These cases and their associated weights are simply removed from the sample, as it is assumed that each of these cases represents a number of out-of-scope persons in the population as reflected by their weight.

In-scope persons maintain the same weight as in the previous step, which is now called wgtT5.

Given the high quality and timeliness of the frame, persons whose scope status is unknown (not enough information was gathered to determine scope status) are considered in-scope and maintain the same weight.

Following this step, the resulting weight, wgtT5, is representative of in-scope persons in the observed population.

6.4 Nonresponse adjusted weight (wgtT6)

During collection, some units result in non-response. This usually occurs when a person refuses to participate in the survey, provides unusable data, or cannot be reached for an interview. Weights of the nonresponding persons were redistributed to responding persons within response homogeneity groups (RHGs), which were formed within each age × gender × population group. To create the RHGs, a scoring method based on logistic regression models was used to determine the propensity of each unit to respond and these response probabilities were used to divide the sample into groups with similar response properties. The regression model used "person" and "household" characteristics from the 2021 Census for the person selected (e.g., income, education level, household composition, limitations to activities of daily living such as emotional, psychological or mental health conditions, etc.). An adjustment factor was calculated within each RHG as follows:

$$\frac{Sum\ of\ weights\ wgtT5\ for\ all\ persons}{Sum\ of\ weights\ wgtT5\ for\ all\ responding\ persons}$$

Weight wgtT5 was multiplied by this adjustment factor to produce weight wgtT6 for the responding persons. Nonresponding persons were dropped from the process at this point.

6.5 Winsorization (wgtT7)

Following the series of adjustments applied to the respondents, some units may end up with extreme weights when compared to other units in the same domain of interest. These units could have a large impact on the variance. To mitigate this, the weight of these outlier units was adjusted downward using a "Winsorization" trimming approach.

6.6 Post-stratification (wgtT8)

The last step necessary to obtain the final MHACS weights is post-stratification. Post-stratification is done to ensure that the sum of the final weights corresponds to the population estimates for age group, gender, and population group domains.

The population estimates are projections in the ten provinces by age based on the 2016 Census counts and counts of birth, death, immigration and emigration since that time. The average of the March to July monthly estimates for each of the age-gender-population group post-strata was used in post-stratification. The weight wgtT7 is adjusted to obtain the final weight wgtT8. Weight wgtT8 corresponds to the final MHACS person-level weight and can be found on the master file with the variable name WTS_M.

6.7 Share Weight

Along with the master file which contains all MHACS respondents, a share file is created which contains only a portion (86%) of the original MHACS respondents. The individuals on this share file have agreed to share their data with certain specified partners. To compensate for the loss of some respondents from the file, the weights of the respondents who agreed to share must be adjusted by the factor:

$$\frac{Sum\ of\ weights\ T6\ for\ all\ MHACS\ respondents}{Sum\ of\ weights\ T6\ for\ all\ MHACS\ respondents\ agreeing\ to\ share\ their\ data}$$

Like the nonresponse adjustments, this factor was calculated within response homogeneity groups where, in this case, respondents with similar estimated propensity to share[4] were grouped together. Weight T6 for respondents who agreed to share was multiplied by the above adjustment factor to produce a share weight. As with the Master weighting process, Winsorization and post-stratification (similar to adjustments T7 and T8) were applied to the share units. The final weight after these adjustments is called WTS_S.

6.8 Bootstrap Weights

For a survey with a complex design, bootstrap weights are used to simplify the process of variance estimation. For the MHACS, 1,000 sets of bootstrap weights were generated, with each set consisting of a bootstrap weight for each respondent. Each set of bootstrap weights was generated based on the initial MHACS design weights and then adjusted in the same way as the survey weights, as described above.

The two phases of sampling embedded in the MHACS design (the 2021 Census of Population sample selection assigning long forms to approximately a quarter of the Canadian population, followed by the sample selection for the MHACS itself) were both taken into account by the generalized bootstrap methodology used for the MHACS.

This method can lead to negative bootstrap weights. This issue is mitigated by applying a transformation to reduce their variability. Users wishing to compute measures of variability using the transformed bootstrap weights must multiply the variability measures by a rescaling factor. **To obtain the correct sampling error estimates, variances must be multiplied by 9. In addition, CVs (square root of the variance divided by the estimate itself) and standard errors must be multiplied by 3.** Most software packages that produce sampling error estimates from bootstrap weights have an option to specify this adjustment factor[5] such that the correct variance estimate is obtained without the need of an extra multiplication step. In software capable of producing sampling error estimates from bootstrap weights, this factor is often specified as the

[4] To model the propensity to share data, we use some of the “person” and “household” characteristics from the 2021 Census considered for the nonresponse adjustment and some key variables taken from of the questionnaire (since sharers are a subset of respondents, we have questionnaire responses for both groups: sharers and non-sharers).

[5] Note that if C is the variance multiplicative factor, some software packages use the parameter k instead, where k = 1-1／√C. In our case, since C=9, then k=0.67.

"Fay adjustment factor". See Section 8.5 for an example of how this can be applied when using SAS version 9.4.

It is critical to use the appropriate multiplicative factor for any estimate of sampling error, such as variance, standard error or CV. Omission of this factor would lead to erroneous results and conclusions.

The 2012 CCHS-MH did not a rescaling factor because a different sampling and weighting methodology was used for that survey. Data users should be aware of this difference when calculating and comparing estimates from the MHACS and the previous surveys in 2012 and 2002 that have similar content. More information about comparing estimates from the MHACS to other Statistics Canada surveys is presented in Section 8.5.

7.0 Data Quality

7.1 Survey errors

Survey errors come from a variety of different sources. They can be classified into two main categories: non-sampling errors and sampling errors.

7.2 Non-sampling errors

Non-sampling errors can be defined as errors arising during the course of virtually all survey activities, apart from sampling. They are present in both sample surveys and censuses (unlike sampling error, which is only present in sample surveys). Non-sampling errors arise primarily from the following sources: non-response, coverage, measurement, and processing.

7.2.1 Non-response

Non-response is both a source of non-sampling error and sampling error. Non-response result from a failure to collect complete information from all units in the selected sample. Non-response is a source of non-sampling error in the sense that non-respondents often have different characteristics from respondents, which can result in biased survey estimates if non-response bias is not fully eliminated through weighting adjustments. The lower the response rate, the higher the risk of bias. Non-response is also a source of sampling error; this is discussed further in Section 7.3.

In total, 39,485 of the selected persons in the MHACS were in-scope for the survey,[6] out of which a response was obtained for 9,861 individuals, resulting in a response rate of 25%. The Table 3 below provides the resulting response rates by Age × Gender as well as by Population group.

Table 3. Response rates by Age group x Gender and by Population group

[6] Among the persons selected, some are not in-scope for the survey. They are, for example, persons who have moved outside of the ten provinces or who are deceased. These units are identified during the data collection, otherwise they would have been excluded before the sample selection. These units are not considered in the calculation of response rates. Persons with unknown scope status are considered to be in-scope.

Age Group	Response Rate		
	Man	Woman	All
15 – 24 years old	18.9%	20.2%	19.5%
25 – 44 years old	24.4%	25.0%	24.7%
45 – 64 years old	25.0%	28.2%	26.6%
65+ years old	31.0%	29.7%	30.4%
All	24.4%	25.5%	25.0%

Population Group	Response Rate
Filipino	22.9%
Black	25.8%
Chinese	21.8%
South Asian	20.0%
Other	27.4%
All	25.0%

7.2.2 Coverage errors

Coverage errors consist of omissions, erroneous inclusions, duplications and misclassifications of units in the survey frame. Coverage errors may cause a bias in the estimates and the effect can vary for different sub-groups of the population.

The MHACS covers the population 15 years of age and over as of March 1, 2022, living in the ten provinces of Canada. Excluded from the survey's coverage are persons living on reserves and other Aboriginal settlements, full-time members of the Canadian Forces, and persons living in collective dwellings, such as institutional residences. Altogether, these exclusions represent less than 3% of the Canadian population aged 12 and over.

7.2.3 Measurement errors

Measurement errors (or sometimes referred to as response errors) occur when the response provided differs from the real value; such errors may be attributable to the respondent, the interviewer, the questionnaire, the collection method or the respondent's record-keeping system. Such errors may be random or they may result in a systematic bias if they are not random.

It is very costly to accurately measure the level of response error and very few surveys conduct a post-survey evaluation. However, interviewer feedback and observation reports usually provide clues as to which questions may be problematic (poorly worded question, inadequate interviewer training, poor translation, technical jargon, no help text available, etc.).

Several measures are taken at Statistics Canada to reduce the level of response error. These measures include questionnaire review and testing using cognitive methods, the use of highly skilled interviewers, extensive training of interviewers with respect to the survey procedures and content, and observation and monitoring of interviewers to detect problems of questionnaire design or misunderstanding of instructions.

7.2.4 Processing errors

Processing error is the error associated with activities conducted once survey responses have been received. It includes all data handling activities after collection and prior to estimation. Like all other errors, they can be random in nature, and inflate the variance of the survey's estimates, or systematic, and introduce bias. It is difficult to obtain direct measures of processing errors and their impact on data quality especially since they are mixed in with other types of errors (non-response, measurement, and coverage).

Data processing of the MHACS was done in a number of steps including data capture, verification, coding, editing, imputation, etc. After each step, data were reviewed by two different reviewers and signed off. As the open-ended questions' write-in responses are coded manually, errors could be introduced at this stage of processing. While there is some subjectivity involved in coding write-in responses and thus a general risk of miscoding, these risks were mitigated through the use of quality assurance testing and the use of reference files provided by Subject Matter Experts at Statistics Canada to ensure consistency across surveys.

7.3 Sampling errors

Sampling error is defined as the error that results from estimating a population characteristic by measuring a portion of the population rather than the entire population. For probability sample surveys, methods exist to calculate sampling error. These methods derive directly from the sample design and method of estimation used by the survey.

The most commonly used measure to quantify sampling error is sampling variance. Sampling variance measures the extent to which the estimate of a characteristic from different possible samples of the same size and the same design differ from one another. For sample designs that use probability sampling, the magnitude of an estimate's sampling variance can be estimated.

Factors affecting the magnitude of the sampling variance include:

1. The variability of the characteristic of interest in the population: the more variable the characteristic in the population, the larger the sampling variance.
2. The size of the population: in general, the size of the population only has an impact on the sampling variance for small to moderate sized populations.
3. The response rate: the sampling variance increases as the sample size decreases. Since non-respondents effectively decrease the size of the sample, non-response increases the sampling variance. This is an issue when the response rate is lower than the anticipated response rate used to inflate the initial sample size.
4. The sample design and method of estimation: some sample designs are more efficient than others in the sense that, for the same sample size and method of estimation, one design can lead to smaller sampling variance than another.

The MHACS sample design includes oversampling of population groups which makes it possible to carry out more disaggregated and intersectional analysis within these groups in particular.

However, this makes the sample design less efficient for estimates that include the whole population together.

The standard error of an estimator is the square root of its sampling variance. This measure is easier to interpret since it provides an indication of sampling error using the same scale as the estimate whereas the variance is based on squared differences.

The coefficient of variation (CV) of an estimate is a relative measure of the sampling error. It is defined as the estimate of the standard error divided by the estimate itself, usually expressed as a percentage (10% instead of 0.1). It is very useful for measuring and comparing the sampling error of quantitative variables with large positive values. However, it is not recommended for estimates such as proportions, estimates of change or differences, and variables that can have negative values.

It is considered a best practice at Statistics Canada to report the sampling error of an estimate through its 95% confidence interval. The 95% confidence interval of an estimate means that if the survey were repeated over and over again, then the confidence interval would cover the true population value 95% of the time (or 19 times out of 20). Section 8.5 provides information on constructing confidence intervals.

7.4 Bias Evaluation

In order to assess the potential impact of the low response rates for the MHACS an evaluation of the potential for bias was conducted at the national level. Population estimates from the 2021 Census of Population were used as benchmarks when evaluating the extent of potential bias in this survey. That is, estimates of 2021 Census proportions using responses and weights from individuals on the MHACS frame (the “benchmarks”) were compared to the same estimates at various stages of the MHACS weight adjustment process using weights as listed in Diagram A. A total of 31 person-level categorical variables were used in the evaluation, including highest education level, age, marital status, self-assessed difficulties experienced in daily living and employment status. Over 100 distinct values and groupings of values were used to compare the estimates of proportions.

The potential for bias during sample selection was minimal, given the random nature of the selection. After adjusting unit weights to account for the selection, it was confirmed that there was little difference between the benchmarks and the estimates using only the sampled units. The potential for bias due to non-response is much greater. However, this potential is greatly reduced by the non-response weight adjustment. Variables selected to the model described in section 6.4 were not only correlated to response likelihood, but also the value of the survey variable MHPFY (Any Selected Disorder – 12-Month Diagnosis). Therefore, the members of a particular response homogenous group were not only likely to be similar to each other in their tendency to respond, but also with respect to their value of MHPFY. By ensuring that non-respondents in these groups transferred their weights respondents who are similar in this regard, the weighted response data is more likely to remain representative of the larger population by this measure.

Almost ¾ of the 95% confidence intervals around the estimates computed from the MHACS final weights and corresponding bootstrap weights contain the corresponding benchmark within their range. For the remaining estimates, all of them differ from their corresponding Census benchmarks by an absolute value less than 2%. Table 4 shows the proportion of estimates calculated using the MHACS final weights within 0.5, 1, or 2 percentage points from the Census benchmark.

Table 4. Comparison of MHACS estimates and Census benchmark

Difference between MHACS estimates and Census benchmark	Proportion

Within 0.5%	64.3%
Within 1%	85.4%
Within 2%	100%

8.0 Guidelines for tabulation, analysis and release

This chapter outlines the guidelines to which users should adhere when tabulating, analyzing, publishing or otherwise releasing any data derived from the survey microdata. With the aid of these guidelines, users of microdata should be able to produce the same figures as those produced by Statistics Canada and, at the same time, will be able to develop currently unpublished figures in a manner consistent with these established guidelines. Methods to measure precision and quality of estimates are also described along with release guidelines to help users decide when an estimate should be included in a publication.

8.1 Rounding guidelines

To ensure that estimates for publication or other use derived from the data files (master or share) correspond to those produced by Statistics Canada, users are urged to adhere to the following guidelines regarding the rounding of such estimates:

a) Estimates in the main body of a statistical table are to be rounded using the normal rounding technique. In normal rounding, if the first or only digit to be dropped is 0 to 4, the last digit to be retained is not changed. If the first or only digit to be dropped is 5 to 9, the last digit to be retained is raised by one.

b) Marginal sub-totals and totals in statistical tables are to be derived from their corresponding unrounded components and then are to be rounded themselves using normal rounding.

c) Averages, rates, percentages, proportions and ratios are to be computed from unrounded components (i.e. numerators and/or denominators) and then are to be rounded themselves using normal rounding.

d) Sums and differences are to be derived from their corresponding unrounded components and then are to be rounded themselves using normal rounding.

e) In instances where, due to technical or other limitations, a rounding technique other than normal rounding is used resulting in estimates to be published or otherwise released which differ from corresponding estimates published by Statistics Canada, users are urged to note the reason for such differences in the publication or release document(s).

f) Under no circumstances are unrounded estimates to be published or otherwise released by users. Unrounded estimates imply greater precision than actually exists.

8.2 Sample weighting guidelines for tabulation

The sample design used for this survey is not self-weighting. That is to say, the sampling weights are not identical for all individuals in the sample. When producing simple estimates, including the production of ordinary statistical tables, users **must** apply the proper sampling weight. If proper weights are not used, the estimates derived from the microdata files cannot be considered to be representative of the survey population and will not correspond to those produced by Statistics Canada.

Users should also note that some software packages might not allow the generation of estimates that exactly match those available from Statistics Canada, because of the treatment of the weight

field. If options are available, users should ensure that they specify that the weight is a sample weight rather than a frequency weight.

8.3 Guidelines for statistical analysis

The MHACS is based on a stratified sample and cross-sectional design. Using data from such complex surveys presents problems to analysts because the survey design and the selection probabilities affect the estimation and variance calculations that should be used. The use of the survey weights is the first step to ensuring that the proper results are obtained.

While many analysis procedures found in statistical packages allow for weights to be used, the meaning or definition of the weight in these procedures can differ from what is appropriate in a sample survey framework, with the result that while in many cases the estimates produced by the package are correct, the variances that are calculated are almost meaningless.

To ensure that results from a complex design are meaningful, the weights must be correctly interpreted by the software package and the proper variance estimation techniques must be used. For example, in SAS, PROC SURVEYMEANS would provide results that are more appropriate as the bootstrap methodology is used for the variance estimates than the results produced from PROC MEANS. PROC MEANS can use the survey weights to correctly adjust estimates, considering the unequal weighting design, but it tends to underestimate the variances in not using the bootstrap weights and assuming the design is simple random sampling.

It is recommended that data users use software packages and functions that can incorporate both sample weights and bootstrap weights while performing their analyses. Guidelines for the variance estimation, constructing confidence intervals and comparing estimates from MHACS to estimates from a survey with a different design are provided in Section 8.5.

8.4 Release guidelines for quality

Before releasing and/or publishing any estimates, analysts should consider the quality level of the estimate. While data quality is affected by both sampling and non-sampling errors, this section covers quality in terms of sampling error. It is considered a best practice at Statistics Canada to report the sampling error of an estimate through its 95% confidence interval (CI). The confidence interval should be released with the estimate, in the same table as the estimate. In addition to the confidence intervals, estimates are categorized into one of three release categories:

Category A
The estimate and confidence interval can be released with no warning. Data users should use the 95% confidence interval to assess whether the quality of the estimate is sufficient. Note that the ‘A’ is not a quality indicator; it should not be released with the estimate.

Category E
The estimate and confidence interval should be flagged with the letter E (or some similar identifier) and accompanied by a quality warning to use the estimate with caution. Data users should use the 95% confidence interval to assess whether the quality of the estimate is sufficient.

Category F
The estimate and confidence interval are not recommended for release. They are deemed of such poor quality, that they are not fit for any use; they contain a very high level of instability, making them unreliable and potentially misleading. If analysts insist on releasing estimates of poor quality, even after being advised of their accuracy, the estimates should be accompanied by a disclaimer. Analysts should acknowledge the warnings given and undertake not to disseminate, present or report the estimates, directly or indirectly, without this disclaimer. The estimates should be flagged with the letter F (or some similar identifier) and the following warning should accompany the estimates and confidence intervals: “Please be warned that these estimates and

confidence intervals [flagged with the letter F] do not meet Statistics Canada's quality standards. Conclusions based on these data will be unreliable, and may be invalid."

The rules for assigning an estimate to a release category depends on the type of estimate.

Release Rules for Estimated Proportions and Estimated Counts

Estimated proportions and estimated counts are computed from binary variables. Estimated counts are estimates of the total number of persons/households with a characteristic of interest; in other words, they are the weighted sum of a binary variable (e.g., estimated number of immigrants). Estimated proportions are estimates of the proportion of persons/households with a characteristic of interest (e.g., estimated proportion of immigrants in the general population). Estimated counts and proportions can also be computed from categorical variables: that is, estimates of the number or proportion of persons/household who belong to a category.

The release rules for estimated proportions and estimated counts are based on sample size. Table 5 provides the release rules for the MHACS, for all estimated proportions and counts.

The size of the domain of individuals who contribute to the estimate is denoted as n. For Table 5, n denotes one of the following:

A) the unweighted count of respondents in the denominator (not the numerator) of the estimated proportion

B) the unweighted count of respondents with nonzero values that contribute to the estimated count.

One of the objectives for MHACS design was to support quality estimates within the four largest individual population groups. This resulted in a stratified sampling design with a disproportionate allocation of the sample to these population groups relative to their prevalence in the population, which is less efficient than a simple random sample for producing estimates for domains that combine these population groups. Due to this, combining different population groups requires a higher unweighted count for dissemination compared to the individual groups to ensure that estimates are of sufficient quality for dissemination.

Example 1: Combining all population group categories, including the catch-all group 'Other', perhaps to disseminate national estimates or estimates by age group and/or by gender, would need 50 respondents to disseminate with a warning. This is greater than the count needed for individual population groups.

It is important to understand the concept of numerators and denominators when interpreting the release rules below for proportions.

Example 2: To estimate the **proportion** of Chinese individuals who perceive their mental health to be excellent, then the denominator in this proportion is the sum of the weights of Chinese respondents. We would then apply Rule 1.1 in Table 5 to the number of records contributing to the **denominator**, and so a minimum of 40 Chinese respondents are required to disseminate an estimate of the proportion of Chinese individuals who perceive their mental health to be excellent.

To produce an estimate of the **count** of Chinese individuals who perceive their mental health to be excellent, Rule 1.1 would be applied to this count (i.e., ensure that there are at least 40 Chinese respondents who perceive their mental health to be excellent.)

For users of the master or the share files, it is required that there be at least 10 observations with the characteristic of interest. If there are fewer than 10 observations in the numerator, the weighted estimate should not be released regardless of the size (n) of the domain.

Table 5. Quality release rules for proportions and counts

Rule	Domain	Disseminate without a warning	Warning to use with caution (E)	Do not disseminate (F)
1.1	Population group and below (The n individuals who contribute to the estimate are within only one of the population groups)	n ≥ 80	40 ≤ n < 80	n < 40
1.2	Otherwise (i.e. this domain contains a mix of population groups, for example "men 65 years old or older")	n ≥ 100	50 ≤ n <100	n < 50

Release Rules for Means, Medians, and Totals of Quantitative Variables

The release rules for the estimated means, medians, and totals of quantitative variables or amounts are based on the sample size and on the coefficient of variation (CV) of the estimate. Table 6 provides the release rules for the MHACS for different scenarios.

The size of the domain of individuals who contribute to the estimate is denoted as n. For Table 6, n denotes one of the following:

A) the unweighted count of respondents that contribute to the estimated mean or median including values of zero

B) the unweighted count of respondents with non-zero values that contribute to the estimated total

As noted in the previous subsection, one of the objectives for MHACS design was to support quality estimates within the four largest individual population groups. Due to this, combining different population groups requires a higher unweighted count for dissemination compared to the individual groups to ensure that estimates are of sufficient quality for dissemination.

Example 3: In the case of releasing an estimate for persons in Southeast Asian population group, Rule 1.1 applies again as it did in the previous subsection. If there were 80 persons in Southeast Asian population group and the CV was 20%, the estimate may be disseminated without warning. However, if the CV was 40%, then the estimate should be disseminated with a warning to use with caution.

For users of the master or the share files, it is required that there be at least 10 observations with the characteristic of interest. If there are fewer than 10 observations in the numerator, the weighted estimate should not be released regardless of the size (n) of the domain and the value of the coefficient of variation for this estimate.

Table 6. Quality release rules for means and totals

Rule	Domain	Disseminate without a warning	Warning to use with caution (E)	Do not disseminate (F)

2.1	Population group and below (The n individuals who contribute to the estimate are within only one of the population groups)	n ≥ 80 and CV ≤ 25%	Otherwise	n < 40 or CV > 50%
2.2	Otherwise (i.e. this domain contains a mix of population groups, for example "men 65 years old or older")	n ≥ 100 and CV ≤ 25%	Otherwise	n < 50 or CV > 50%

Release Rules for Differences

To assign one of the two release categories defined in Section 8.4 to an estimated difference between two estimates, the analyst must first determine the release category of each of the two estimates using the rules described above. Next, the release category of the estimated difference or the estimate of change is assigned the lower quality release category of the two estimates. This can be specified as follows:

- If one or both estimates are category F estimates, then assign the estimated difference to category F and is not recommended for release.
- Otherwise, If one or both estimates are category E estimates, then assign the estimated difference to category E.
- If both estimates are category A estimates, then assign the estimated difference to category A.

Additional Rules Regarding Confidence intervals

The above release rules should suppress most estimates and corresponding confidence intervals of poor quality. However, there are two additional conditions that indicate that a confidence interval is of poor quality. An estimate and its confidence interval should be assigned to release category F if either of the following two conditions are true:

- The lower bound of the 95% confidence interval is equal to the upper bound of the interval; in other words, the confidence interval is of length zero. (Exceptions are if the estimate corresponds to a post-stratification control total.)
- The lower bound or upper bound of the 95% confidence interval is not a plausible value for the estimate. For example, the lower bound for an estimated proportion is negative.

8.5 Guidelines for Statistical Analysis, Variance Estimation and Constructing Confidence intervals

To measure the sampling error of estimates, variance estimates need to be calculated and confidence intervals need to be constructed. The MHACS uses a complex sample design and estimation method, so there is no simple formula for calculating variance estimates. The survey therefore uses a generalized version of a resampling method called the bootstrap. One thousand sets of bootstrap weights were generated, named BSW1-BSW1000. Essentially, the variance is estimated by calculating the value of the desired estimate using each set of bootstrap weights and then measuring the variability between these 1000 bootstrap estimates.

Statistical packages for statistical analysis and variance estimation

For MHACS, it is necessary to use bootstrap weights to compute correct estimates of the variance. Several statistical software programs or packages are specifically designed for analyses of data from complex survey designs and can compute variance estimates using bootstrap weights. These include for example SUDAAN, WesVar, Stata and newer versions of SAS.

Other standard and/or older statistical analysis software packages, including SSPS and versions of SAS prior to version 9.2, do not have an integrated procedure to calculate variance estimates from bootstrap weights when using data based on a complex survey design like MHACS. These packages should not be used to calculate variance estimates, to construct confidence intervals, nor to conduct statistical tests (significance tests, regression analysis, etc.).

SAS version 9.2 and above can calculate variances from bootstrap weights, as well as other types of replicate weights such as Jackknife and Balanced Repeated Replication (BRR) weights. There are also several procedures, such as regression and logistic regression, that accommodate replicate weights. Confidence intervals for medians using replicate weights are only available in SAS version 9.3 and above.

It should be noted that software packages that do not explicitly support bootstrap weights but do support the BRR method can be used with bootstrap weights. While the bootstrap and BRR methods differ in the way in which the replicate weights are built, once the replicate weights are produced, the two methods use a similar formula to compute variance estimates. For more information on the relationship between the bootstrap and the BRR method, please refer to Phillips (2004).

Confidence intervals

The most commonly used method of constructing 95% confidence intervals is the Wald interval, which is of the form $\hat{y} \pm 1.96\sqrt{\hat{\text{var}}(\hat{y})}$ for an estimate $\hat{y}$ with estimated variance $\hat{\text{var}}(\hat{y})$, where the estimated variance is calculated using the multiplicative factor (phi) as described in section 6.8. Wald intervals assume that the sampling distribution of $\hat{y}$ is approximately normal. For proportions, the normality assumption is known to break down for small sample sizes and for proportions near zero or one. Three alternative methods of constructing confidence intervals are therefore recommended for proportions: the modified Wilson interval, the modified Clopper-Pearson interval and the logit interval (see Korn and Graubard, 1998; Liu and Kott, 2009). There are options in SAS and SUDAAN to produce confidence intervals using these alternative methods.
The examples below show how alternative methods of constructing confidence intervals are specified for proportions in SAS and SUDAAN. **It is extremely important to use the appropriate multiplicative factor for any estimate of sampling error such as variance, standard error or CV.** Refer to section 6.8 about details on how to use the bootstrap weights and the multiplicative factor in variance calculation.

1. SAS, modified Wilson confidence intervals:
```
PROC SURVEYFREQ
DATA=…. VARMETHOD=BRR (FAY=0.67);
WEIGHT WTMP;
REPWEIGHTS BSW1-BSW1000;
TABLES …. / CL (TYPE=WILSON ADJUST=NO TRUNCATE=YES)
```

2. SUDAAN, modified Clopper-Pearson confidence intervals:
```
PROC CROSSTAB
DATA=…. DESIGN=BRR SMCONF=50;
WEIGHT WTMP;
REPWGT BSW1-BSW1000 / ADJFAY=9;
TABLES ...;
```

For both Examples 1 and 2, variance estimates can be obtained by specifying either an OUTPUT or PRINT statement after the TABLES statement.

Comparison of estimates <u>between</u> surveys

When comparing estimates, confidence intervals and hypothesis tests are often considered to make an inference if there is enough evidence that the difference in estimates is statistically significate. For MHACS, it is important to remember that, because bootstrap weights for MHACS were created using a different methodology than for most other health surveys in Statistics Canada, the variance adjustment factor **<u>must</u>** be applied when estimating variance for MHACS estimates, while most other health surveys such adjustment **<u>must not</u>** be applied.

There is no statistical package, e.g. SUDAAN, that is able to create a confidence interval or perform a hypothesis test for an estimate of a difference in one step. The following steps are therefore recommended when estimating differences of proportions (or aggregates) between surveys:

1) Estimate a proportion from MHACS and its variance using one of the methods mentioned above:
   $\hat{X}_{MHACS}$ and $\widehat{var}(\hat{X}_{MHACS})$
2) Estimate a proportion from another survey and its variance:
   $\hat{X}_2$ and $\widehat{var}(\hat{X}_2)$
3) Estimate the difference between proportions as $\hat{d} = \hat{X}_{MHACS}$ - $\hat{X}_2$
4) Estimate the variance of the estimate of the difference. When two sample designs are independent, the variance of a difference between two estimates is approximately equal to the sum of the variances of each estimate. That is, the variance of the difference can be estimated as: $\widehat{var}(\hat{d}) = \widehat{var}(\hat{X}_{MHACS}) + \widehat{var}(\hat{X}_2)$.
5) Construct a 95% confidence interval for the difference: $(\hat{d} - 1.96\sqrt{\widehat{var}(\hat{d})}, \hat{d} + 1.96\sqrt{\widehat{var}(\hat{d})})$
6) A hypothesis test made to assess if the population proportions (or other characteristics) from two surveys are the same can be done by performing a Z-test. The z-score for the Z-test can be calculated by the following formula:
   $z = \frac{\hat{d}}{\sqrt{\widehat{var}(\hat{d})}}$

References

Korn, E.L., and Graubard, B.I. (1998). “Confidence Intervals for Proportions With Small Expected Number of Positive Counts Estimated From Survey Data”. Survey Methodology, 24, 193-201.

Liu, Y.K. and Kott, P.S. (2009). “Evaluating Alternative One-Sided Coverage Intervals for a Proportion”. Journal of Official Statistics, Vol. 25, No. 4, 569-588.

Phillips, O. (2004). “Using bootstrap weights with WesVar and SUDAAN” (Catalogue no. 12-002-X20040027032) in The Research Data Centres Information and Technical Bulletin, Chronological index, Fall 2004, vol.1 no. 2 Statistics Canada, Catalogue no. 12-002-XIE.

Appendix A:

Comparison of 2012 CCHS - Mental Health, 2002 CCHS - Mental Health and Well-Being, and 2022 Mental Health and Access to Care Survey

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
SAMPLEID	SAMPLEID	MASTERID	Randomly generated sequence number for master file	Same	Same		
VERDATE	VERDATE	VERDATE	Date of file creation	Same	Same		
REFPER	REFPER	REFPER	Reerence period	Same	Same		
	ADM_N12	ADM_LAN	Collection language	Same			
GEOBDCMA	GEODCMA1	GEODVCMA	2021 Census Metropolitan Area - (D)	Modified	Modified	Updated to account for 2021 Census geography	Updated to account for 2011 Census geography
GEOB_PRV	GEO_PRV	GEO_PRV	Province of residence of respondent	Same	Same		
GEOB_PC	GEODPC	GEODVPC	Postal code - (D)	Same			
	GEODUR	GEODVUR	Urban and Rural Areas - (D)	Modified	Added	Derived variable conditions changed, concept unchanged. See DV specifications.	
GEOBDUR2	GEODUR2	GEODVUR2	Urban and Rural Areas - 2 levels - (D)	Same	Modified		
		GEODVPSZ	Population centre or rural area classification - (D)	Added			
		CAF_01	Full time member of the regular CAF	Added			
DHHB_MS	DHH_MS	MS_01	Marital status	Same	Same		
DHHB_AGE	DHH_AGE	DV_AGE	Age	Same	Same		
DHHB_SEX	DHH_SEX	GDR_005	Sex	Same	Same		
		GENDER	Gender (2)	Added			
		GDRDCTNB	Cisgender, transgender and non-binary	Added			
GENB_01	GEN_01	GEN_01	Self-perceived health	Same	Same		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
GENB_02	GEN_02	GEN_02A	Self-perceived health compared to one year ago	Same	Same		
SCRB_082	SCR_082	GEN_08B	Self-perceived mental health	Same	Same		
		GEN_08C	Self-perceived mental health compared to one year ago	Added			
GENB_04	GEN_04	GEN_02A1	Frequency - trouble sleeping	Same	Same		
	GEN_02A2	GEN_02B	Satisfaction with life in general - 11 point scale	Same	Added		
GENB_07	GEN_07	GEN_07	Perceived life stress	Same	Same		
GENB_08	GEN_08	GEN_08	Worked at job or business	Same	Same		
GENB_09	GEN_09	GEN_09	Self-perceived work stress	Same	Same		
GENB_10	GEN_10	GEN_10	Sense of belonging to local community	Same	Same		
GENBDHDI	GENDHDI	GENDHDI	Perceived health - (D)	Same	Same		
	GENGSWL	GENGSWL	Satisfaction with life in general - (G)	Same	Added		
SCRB_21	SCR_21	SCR_21	Screener: felt sad/empty/depressed for several days - life	Same	Same		
SCRB_22	SCR_22	SCR_22	Screener: was very discouraged several days or longer - life	Same	Same		
SCRB_23	SCR_23	SCR_23	Screener: lost interest in things for several days - life	Same	Same		
SCRB_24	SCR_24	SCR_24	Screener: had mania-like period lasting several days - life	Same	Same		
SCRB_25	SCR_25	SCR_25A	Screener: very irritable/grumpy/ for several days - life	Same	Same		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
SCRB_25A	SCR_25A	SCR_25B	Screener: so irritable started arguments/shouted/... - life	Same	Same		
SCRB_26	SCR_26	SCR_26A	Screener: worried about things more than most people - life	Same	Same		
SCRB_26A	SCR_26A	SCR_26B	Screener: more nervous/anxious than most people - life	Same	Same		
SCRB_26B	SCR_26B	SCR_26C	Screener: 6 months when anxious/worried most days - life	Same	Same		
SCRB_29		SCR_29A	Screener: intense fear or shyness with people - life	Added			
SCRB_29A		SCR_29B	Screener: intense fear or shyness in front of a group - life	Added			
SCRB_291		SCR_29C	Screener: upset or nervousness in front of a group - life	Added			
SCRB_292		SCR_29D	Screener: stayed away because of fear in front of a group - life	Added			
SCRB_293		SCR_29E	Screener: think the fear was much stronger than it should have been	Added			
SCRBDMEN	SCRDMEN	SCRDMEN	Self-rated mental health - (D)	Same	Same		
SCRBDDEP	SCRDDEP	SCRDDEP	Respondent screened in depression module - (F)	Same	Same		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
SCRBDMIA	SCRDMIA	SCRDMIA	Respondent screened in mania module - (F)	Same	Same		
	SCRDGAD	SCRDGAD	Respondent screened in anxiety module - (F)	Same	Added		
SCRBDSOP		SCRDSOP	Respondent screened in social phobia module - (F)	Added			
CCCB_031	CCC_031	CCC_031	Has asthma	Same	Same		
CCCB_051	CCC_051	CCC_051	Has arthritis	Same	Modified		Question reworded
CCCB_061	CCC_061	CCC_061	Has back problems excluding fibromyalgia and arthritis	Same	Same		
		CCC_062	Has fibromyalgia	Added			
CCCB_071	CCC_071	CCC_071	Has high blood pressure	Same	Same		
	CCC_072	CCC_072	Ever diagnosed with high blood pressure	Same	Added		
CCCB_081	CCC_081	CCC_081	Has migraine headaches	Same	Same		
CCCB_091	CCC_091	CCC_091	Has chronic bronchitis/ emphysema/ COPD	Same	Modified		Combination of two questions in the 2002 survey
CCCB_101	CCC_101	CCC_101	Has diabetes	Same	Same		
CCCB_121	CCC_121	CCC_121	Has heart disease	Same	Same		
CCCB_131	CCC_131	CCC_131	Has cancer	Same	Same		
	CCC_132	CCC_132	Ever had cancer	Same	Added		
CCCB_171	CCC_171	CCC_171A	Has bowel disorder/Crohnâ€™s Disease/ulcerative colitis	Same	Same		
	CCC_17A	CCC_171B	Type of bowel disease	Same	Added		
CCCB_251	CCC_251	CCC_251	Has chronic fatigue syndrome	Same	Same		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
CCCB_261	CCC_261	CCC_261	Suffers from multiple chemical sensitivities	Same	Same		
CCCB_271	CCC_271	CCC_271A	Has schizophrenia	Same	Same		
	CCC_271B	CCC_271B	Ever diagnosed with schizophrenia	Same	Added		
CCCB_281	CCC_281	CCC_272A	Has any other psychosis	Same	Same		
	CCC_281B	CCC_272B	Ever diagnosed with any other psychosis	Same	Added		
	CCC_280	CCC_280A	Has a mood disorder	Same	Added		
	CCC_28AA	CCC280BA	Has mood disorder - depression	Same	Added		
	CCC_28AB	CCC280BB	Has mood disorder - bipolar disorder	Same	Added		
	CCC_28AC	CCC280BC	Has mood disorder - mania	Same	Added		
	CCC_28AD	CCC280BD	Has mood disorder - dysthymia	Same	Added		
	CCC_28AE	CCC280BE	Has mood disorder - other	Same	Added		
	CCC_290	CCC_290A	Has an anxiety disorder	Same	Added		
	CCC_29AA	CCC290BA	Has anxiety disorder - phobia	Same	Added		
	CCC_29AB	CCC290BB	Has an anxiety disorder - obsessive-compulsive disorder	Same	Added		
	CCC_29AC	CCC290BC	Has an anxiety disorder - panic disorder	Same	Added		
	CCC_29AD	CCC290BD	Has an anxiety disorder - other	Same	Added		
CCCB_311	CCC_311	CCC_311	Has post-traumatic stress disorder	Same	Same		
CCCB_331	CCC_331	CCC_331	Has a learning disability	Same	Same		
CCCB_332	CCC_332	CCC_332	Has Attention Deficit Disorder	Same	Modified		Question reworded

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
CCCB_341	CCC_341	CCC_341	Has eating disorder such as anorexia or bulimia	Same	Same		
CCCB_901	CCC_901	CCC_901	Has other long-term physical or mental health condition	Same	Same		
CCCBF1	CCCF1	CCCF1	Has a chronic condition - (F)	Same	Modified		Conditions changed in DV specifications
	CCCDHBP	CCCDHBP	Has or previously diagnosed with high blood pressure (D)	Same	Added		
	CCCDCNCR	CCCDCNCR	Has or previously diagnosed with cancer (D)	Same	Added		
	CCCDSCZO	CCCDSCZO	Has or previously diagnosed with schizophrenia (D)	Same	Added		
	CCCDPSYC	CCCDPSYC	Has or previously diagnosed with psychosis (D)	Same	Added		
	HUP_01	HUP_28	Usually free of pain or discomfort	Same	Added		
	HUP_02	HUP_29	Pain and discomfort - usual intensity	Same	Added		
	HUP_03	HUP_30	Pain and discomfort - number of activities prevented	Same	Added		
	HUPDPAD	HUPDPAD	Pain (function code) - (D)	Same	Added		
	PHS_01	PHS_01	Participate in physical activity - freq 7 d	Same	Added		
	PHS_02	PHS_02	Time spent - each exercise occasion	Same	Added		
	PHSFPPA	PHSFPPA	Flag: moderate/vigorous physical activity - 7 days (F)	Same	Added		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
	PHSDAPA	PHSDAPA	Moderate/vigorous physical activity - average/hours - 7 D (D)	Same	Added		
		IM_10A	COVID-19 impacts - Loss of job or income	Added			
		IM_10B	COVID-19 impacts - Financial difficulties	Added			
		IM_10C	COVID-19 impacts - Accessing childcare services difficult	Added			
		IM_10D	COVID-19 impacts - Accessing medications difficult	Added			
		IM_10E	COVID-19 impacts - Accessing health care services difficult	Added			
		IM_10F	COVID-19 impacts - Diagnosed with COVID-19	Added			
		IM_10G	COVID-19 impacts - Hospitalized due to COVID-19	Added			
		IM_10H	COVID-19 impacts - Severe illness of a loved one	Added			
		IM_10I	COVID-19 impacts - Death of a loved one	Added			
		IM_10J	COVID-19 impacts - Feelings of loneliness or isolation	Added			
		IM_10K	COVID-19 impacts - Emotional distress	Added			
		IM_10L	COVID-19 impacts - Physical health problems	Added			
		IM_10M	COVID-19 impacts - Relationships	Added			

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
			challenges with household members				
		IM_10N	COVID-19 impacts - Other	Added			
		IM_10O	COVID-19 impacts - None of the above	Added			
	PMH_01	PMH_01	Pos ment hlth - happy - freq 1 mo	Same	Added		
	PMH_02	PMH_02	Pos ment hlth - interested in life - freq 1 mo	Same	Added		
	PMH_03	PMH_03	Pos ment hlth - satisfied with life - freq 1 m	Same	Added		
	PMH_04	PMH_04	Pos ment hlth - had somethng to contribut to soc - freq 1 mo	Same	Added		
	PMH_05	PMH_05	Pos ment hlth - belong to a community - freq 1 mo	Same	Added		
	PMH_06	PMH_06	Pos ment hlth - society becoming a better place - freq 1 mo	Same	Added		
	PMH_07	PMH_07	Pos ment hlth - people basically good - freq 1 mo	Same	Added		
	PMH_08	PMH_08	Pos ment hlth - society makes sense - freq 1 mo	Same	Added		
	PMH_09	PMH_09	Pos ment hlth - liked most parts of personality - freq 1 mo	Same	Added		
	PMH_10	PMH_10	Pos ment hlth - good at managing daily life - freq 1 mo	Same	Added		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
	PMH_11	PMH_11	Pos ment hlth - have warm/trusting relationships - freq 1 mo	Same	Added		
	PMH_12	PMH_12	Pos ment hlth - challenged to become better - freq 1 mo	Same	Added		
	PMH_13	PMH_13	Pos ment hlth - confident to express opinions - freq 1 mo	Same	Added		
	PMH_14	PMH_14	Pos ment hlth - life has meaning - freq 1 mo	Same	Added		
	PMHDHEM	PMHDHEM	High emotional well-being - (D)	Same	Added		
	PMHDLEM	PMHDLEM	Low emotional well-being - (D)	Same	Added		
	PMHDHFU	PMHDHFU	High positive functioning - (D)	Same	Added		
	PMHDLFU	PMHDLFU	Low positive functioning - (D)	Same	Added		
	PMHDCLA	PMHDCLA	Positive mental health - classification - (D)	Same	Added		
	PMHDSCR	PMHDSCR	Positive mental health - continuous score - (D)	Same	Added		
STSB_1	STS_1	STS_1	Self-perceived ability to handle unexpected problem	Same	Same		
STSB_2	STS_2	STS_2	Self-perceived ability to handle day-to-day demands	Same	Same		
STSB_3	STS_3	STS_3	Most important source of feelings of stress	Same	Same		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
	STS_4	STS_4	Can count on people / deal with stress	Same	Added		
	STS_5	STS_5	Perceived personal ability to deal with stress	Same	Added		
DISB_10A	DIS_10A	DIS_01A	Freq/distress: felt tired out - past month	Same	Same		
DISB_10B	DIS_10B	DIS_01B	Freq/distress: felt nervous - past month	Same	Same		
DISB_10C	DIS_10C	DIS_01C	Freq/distress: so nervous nothing calms down - past month	Same	Same		
DISB_10D	DIS_10D	DIS_01D	Freq/distress: felt hopeless - past month	Same	Same		
DISB_10E	DIS_10E	DIS_01E	Freq/distress: felt restless / fidgety - past month	Same	Same		
DISB_10F	DIS_10F	DIS_01F	Freq/distress: could not sit still - past month	Same	Same		
DISB_10G	DIS_10G	DIS_01G	Freq/distress: felt sad / depressed - past month	Same	Same		
DISB_10H	DIS_10H	DIS_01H	Freq/distress: depressed/nothing cheers - past month	Same	Same		
DISB_10I	DIS_10I	DIS_01I	Freq/distress: felt everything effort - past month	Same	Same		
DISB_10J	DIS_10J	DIS_01J	Freq/distress: felt worthless - past month	Same	Same		
DISB_10K	DIS_10K	DIS_01K	Freq/distress: occurrence of feelings - past month	Same	Same		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
DISB_10L	DIS_10L	DIS_01L	Degree of higher occurrence of feelings - past month	Same	Same		
DISB_10M	DIS_10M	DIS_01M	Degree of lower occurrence of feelings - past month	Same	Same		
DISB_10N	DIS_10N	DIS_01N	Interference - feelings in life / activities - past month	Same	Same		
DISBDK6	DISDK6	DISDK6	Distress scale - K6 - past month - (D)	Same	Same		
DISBDDSX	DISDDSX	DISDDSX	Distress scale - K10 - past month - (D)	Same	Same		
DISBDCHR	DISDCHR	DISDCHR	Chronicity of distress/impairment scale - past month (D)	Same	Same		
DEPB_01	DEP_01	DEP_01A	Period - discouraged about how things going - life	Same	Same		
DEPB_01A	DEP_01A	DEP_01A1	Period - lost interest in most things - life	Same	Same		
DEPB_01B	DEP_01B	DEP_01B	Period - lost interest in most things - life	Same	Same		
DEPB_02	DEP_02	DEP_02	Period - lost interest in most things - life	Same	Same		
DEPB_09	DEP_09	DEP_09	2-week period - lost interest in most things - life	Same	Same		
DEPB_12	DEP_12	DEP_12	2-week period - sad and/or discouraged and/or uninterested	Same	Same		
DEPB_16	DEP_16	DEP_16	Daily duration - being sad/discouraged/uninterested - life	Same	Same		
DEPB_17	DEP_17	DEP_17	Emotional distress - intensity - life	Same	Same		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
DEPB_18	DEP_18	DEP_18	Distress so severe nothing cheer up - frequency - life	Same	Same		
DEPB_19	DEP_19	DEP_19	Distress severe/could not carry daily - frequency -life	Same	Same		
DEPB_21	DEP_21	DEP_21	Had depressive-like symptoms during episodes - life	Same	Same		
DEPB_22	DEP_22	DEP_22A	Experienced one particular worst episode - life	Same	Same		
DEPB_22A	DEP_22A	DEP_22A1	Worst episode started - age	Same	Same		
DEPB_22B1	DEP_22B1	DEP_22B	Worst episode - duration	Same	Same		
DEPB_22C1	DEP_22C1	DEP_22C	Worst episode - unit: days/weeks/months/years	Same	Modified		Follow-up questions consolidated for ease of programming.
DEPB_23A	DEP_23A	DEP_23A	Last bad episode - age	Same	Same		
DEPB_23B1	DEP_23B1	DEP_23B	Last bad episode - duration	Same	Same		
DEPB_23C1	DEP_23C1	DEP_23C	Last bad episode - unit: days/weeks/months/years	Same	Modified		Follow-up questions consolidated for ease of programming
DEPB_24A	DEP_24A	DEP_24A	Symptom - felt sad / empty / depressed	Same	Same		
DEPB_24B	DEP_24B	DEP_24B	Symptom - felt so sad that nothing could cheer up	Same	Same		
DEPB_24C	DEP_24C	DEP_24C	Symptom - felt discouraged about how things were going	Same	Same		
DEPB_24D	DEP_24D	DEP_24D	Symptom - felt hopeless about future	Same	Same		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
DEPB_24E	DEP_24E	DEP_24E	Symptom - lost interest in almost all things	Same	Same		
DEPB_24F	DEP_24F	DEP_24F	Symptom - felt like nothing was fun	Same	Same		
DEPB_26A	DEP_26A	DEP_26A	Symptom - had much smaller appetite than usual	Same	Same		
DEPB_26B	DEP_26B	DEP_26B	Symptom - had much larger appetite than usual	Same	Same		
DEPB_26C	DEP_26C	DEP_26C	Symptom - gained weight without trying to	Same	Same		
DEPB_261	DEP_261	DEP_26C1	Weight gain - due to physical growth/pregnancy	Same	Same		
DEPB_26D	DEP_26D	DEP_26D	Weight gain - amount	Same	Same		
DEPB_262	DEP_262	DEP_N26D	Weight gain - unit: pounds or kilograms	Same	Same		
DEPB_26E	DEP_26E	DEP_26E	Symptom - lost weight without trying to	Same	Same		
DEPB_263	DEP_263	DEP_26E1	Weight loss - result of diet/physical illness	Same	Same		
DEPB_26F	DEP_26F	DEP_26F	Weight loss - amount	Same	Same		
DEPB_264	DEP_264	DEP_N26F	Weight loss - unit: pounds or kilograms	Same	Same		
DEPB_26G	DEP_26G	DEP_26G	Symptom - had more trouble falling/staying asleep	Same	Same		
DEPB_26H	DEP_26H	DEP_26H	Symptom - slept more than usual	Same	Same		
DEPB_26I	DEP_26I	DEP_26I	Symptom - slept less than usual/still not tired	Same	Same		
DEPB_26J	DEP_26J	DEP_26J	Symptom - felt tired/low in energy	Same	Same		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
DEPB_26K	DEP_26K	DEP_26K	Symptom - had more energy than usual	Same	Same		
DEPB_26L	DEP_26L	DEP_26L	Symptom - talked/moved more slowly than normal	Same	Same		
DEPB_26M	DEP_26M	DEP_26M	Symp - others noticed respondent talked/moved slowly	Same	Same		
DEPB_26N	DEP_26N	DEP_26N	Symptom - restless/jittery/couldn't sit still	Same	Same		
DEPB_26O	DEP_26O	DEP_26O	Symptom - others noticed respondent was restless	Same	Same		
DEPB_26P	DEP_26P	DEP_26P	Symp - thoughts came more slowly than usual	Same	Same		
DEPB_26Q	DEP_26Q	DEP_26Q	Symptom - thoughts jumped/raced through head	Same	Same		
DEPB_26R	DEP_26R	DEP_26R	Symptom - had more trouble concentrating	Same	Same		
DEPB_26S	DEP_26S	DEP_26S	Symptom - unable to make up mind	Same	Same		
DEPB_26T	DEP_26T	DEP_26T	Symptom - lost self-confidence	Same	Same		
DEPB_26U	DEP_26U	DEP_26U	Symptom - felt was not as good as other people	Same	Same		
DEPB_26V	DEP_26V	DEP_26V	Symptom - felt totally worthless	Same	Same		
DEPB_26W	DEP_26W	DEP_26W	Symptom - felt guilty nearly every day	Same	Same		
DEPB_26X	DEP_26X	DEP_26X	Symp - felt irritable/grouchy or in a bad mood	Same	Same		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
DEPB_26Y	DEP_26Y	DEP_26Y	Symptom - felt nervous/anxious most days	Same	Same		
DEPB_26Z	DEP_26Z	DEP_26Z	Symptom - had attacks of intense fear or panic	Same	Same		
DEPB_265	DEP_265	DEP26ZFF	Symptom - could not cope with everyday responsibilities	Added	Same		
DEPB_266	DEP_266	DEP26ZGG	Symptom - wanted to be alone rather than with friends	Added	Same		
DEPB_267	DEP_267	DEP26ZHH	Symptom - felt less talkative than usual	Added	Same		
DEPB_268	DEP_268	DEP26ZII	Symptom - was often in tears	Added	Same		
DEPB_6A	DEP_6A	DEP_26AA	Symptom - thought a lot about own death/death in general	Same	Same		
DEPB_6B	DEP_6B	DEP_26BB	Symptom - thought it would be better if dead	Same	Same		
DEPB_A	SUI_01	SUI_01	Suicide - ideation during worst / bad episode	Same	Same		
DEPB_A1	SUI_02	SUI_02	Suicide - ideation - life	Same	Same		
DEPB_A1A,DEPB_A1B	SUI_03	SUI_03	Suicide - ideation - 12 mo	Same	Same		
DEPB_A2	SUI_03_1	SUI_03_1	Suicide - age when last ideation occurred	Same	Same		
DEPB_B	SUI_04A	SUI_04A	Suicide - plan during worst/bad episode	Same	Same		
	SUI_04B	SUI_04B	Suicide - plan - life	Same	Added		
DEPB_B1C	SUI_05	SUI_05	Suicide - plan - 12 mo	Same	Same		
	SUI_05_1	SUI_05_1	Suicide - age when last plan occurred	Same	Added		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
DEPB_C	SUI_06A	SUI_06A	Suicide - attempt - worst/bad episode	Same	Same		
DEPB_CB,DEPB_C1	SUI_06B	SUI_06B	Suicide - attempt - life	Same	Same		
	SUI_06C	SUI_06C	Suicide - number of attempts - life	Same	Added		
DEPB_C1A	SUI_10	SUI_10	Suicide - attempt - 12 mo	Same	Same		
DEPB_C1D	SUI_10_1	SUI_10_1	Suicide - age when last attempt occurred	Same	Same		
DEPB_C3	SUI_11	SUI_11	Suicide - attempt resulted in injury or poisoning	Same	Same		
DEPB_C4	SUI_12	SUI_12	Suicide - medical attention following most recent attempt	Same	Same		
DEPB_C5	SUI_13	SUI_13	Suicide - hospitalized overnight following recent attempt	Same	Same		
DEPB_A7	SUI_16	SUI_16	Suicide - ideation - consulted professional - 12 m	Same	Same		
DEPB_D8	SUI_19	SUI_19	Suicide - ideation/attempt - consulted professional - 12 m	Same	Same		
DEPB_28	DEP_28	DEP_28A	Interference - work/social life/pers relations - life	Same	Same		
DEPB_281	DEP_281	DEP_28B	Interference - 2 wks+ /work/social life/pers relation - life	Added	Same		
DEPB_28A	DEP_28A	DEP_28C	Interference - unable carry out daily act - life	Same	Same		
DEPB_29A	DEP_29A	DEP_29A	Depression - any episode - physical/med/drug/alc causes	Same	Same		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
DEPB_29B	DEP_29B	DEP_29B	Depression - all episodes - physical/med/drug/alc causes	Same	Same		
DEPB_29CA	DEP_29CA	DEP_29CA	Physical exclusion - exhaustion	Same	Same		
DEPB_29CB	DEP_29CB	DEP_29CB	Physical exclusion - hyperventilation	Same	Same		
DEPB_29CC	DEP_29CC	DEP_29CC	Physical exclusion - hypochondria	Same	Same		
DEPB_29CD	DEP_29CD	DEP_29CD	Physical exclusion - menstrual cycle	Same	Same		
DEPB_29CE	DEP_29CE	DEP_29CE	Physical exclusion - pregnancy/postpartum	Same	Same		
DEPB_29CF	DEP_29CF	DEP_29CF	Physical exclusion - thyroid disease	Same	Same		
DEPB_29CG	DEP_29CG	DEP_29CG	Physical exclusion - cancer	Same	Same		
DEPB_29CH	DEP_29CH	DEP_29CH	Physical exclusion - overweight	Same	Same		
DEPB_29CI	DEP_29CI	DEP_29CI	Physical exclusion - medication (excl illicit drugs)	Same	Same		
DEPB_29CJ	DEP_29CJ	DEP_29CJ	Physical exclusion - illicit drugs	Same	Same		
DEPB_29CK	DEP_29CK	DEP_29CK	Physical exclusion - alcohol	Same	Same		
DEPB_29CL	DEP_29CL	DEP_29CL	Physical exclusion - chemical/serotonin imbalance	Same	Same		
DEPB_29CM	DEP_29CM	DEP_29CM	Physical exclusion - chronic pain	Same	Same		
DEPB_29CN	DEP_29CN	DEP_29CN	Physical exclusion - caffeine	Same	Same		
DEPB_29CO	DEP_29CO	DEP_29CO	Physical exclusion - no specific diagnosis	Same	Same		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
DEPB_29CP	DEP_29CP	DEP_29CP	Physical exclusion - accident/injury	Same	Same		
DEPB_29CQ	DEP_29CQ	DEP_29CQ	Physical exclusion - emotional - social - economic reason	Same	Same		
DEPB_29CR	DEP_29CR	DEP_29CR	Physical exclusion - other - physical causes	Same	Same		
DEPB_30	DEP_30	DEP_30A	Episode ever occurred after someone close died	Same	Same		
DEPB_30A	DEP_30A	DEP_30B	Episodes always occurred after someone close died	Same	Same		
DEPB_31	DEP_31	DEP_31	Number of episodes lasting 2 weeks or longer - life	Same	Same		
DEPB_372	DEP_372	DEP_37	Dep - 1 episode life >= 2 w/brought on: stress/out of blue	Same	Same		
DEPB_38	DEP_38	DEP_38A	Depression - episode - past 12 months	Same	Same		
DEPB_8A	DEP_8A	DEP_38A1	Depression - most recent episode during past 12 months	Same	Same		
DEPB_38B	DEP_38B	DEP_38B	Depression - number of days out of 365 in episode - 12 m	Same	Same		
DEPB_38C	DEP_38C	DEP_38C	Depression - most recent episode - age	Same	Same		
DEPB_39A1	DEP_39A1	DEP_39	Depression - longest episode - duration	Same	Same		
DEPB_39B1	DEP_39B1	DEP_39_1	Dep - longest episode - unit: days/weeks/months/years	Same	Modified		Follow-up questions consolidated for ease of programming.

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
DEPB_531	DEP_531	DEP_53	Dep - episodes >= 2 weeks/ num brought on by stress	Same	Same		
DEPB_64A	DEP_64A	DEP_64A	Symptom - felt cheerful - frequency - 12 m	Same	Same		
DEPB_64B	DEP_64B	DEP_64B	Symptom - felt slowed down - frequency - 12 m	Same	Same		
DEPB_64C	DEP_64C	DEP_64C	Symptom - enjoyed good book/radio/TV - freq - 12 m	Same	Same		
DEPB_65A	DEP_65A	DEP_65A	Symptom - enjoyed things had enjoyed/ freq -12 m	Same	Same		
DEPB_65B	DEP_65B	DEP_65B	Symptom - laughed / saw bright side of things - freq 12 m	Same	Same		
DEPB_65C	DEP_65C	DEP_65C	Symptom - took interest in physical appearance - freq -12 m	Same	Same		
DEPB_65D	DEP_65D	DEP_65D	Symptom - looked forward to enjoying things/freq 12 m	Same	Same		
DEPB_66A	DEP_66A	DEP_66A	Dep - level of interference - home responsibilities - 12 m	Same	Same		
DEPB_661	DEP_661	DEP_66B1	Dep - level of interference -ability to attend school - 12 m	Same	Same		
DEPB_662	DEP_662	DEP_66B2	Dep - level of interference -ability to work at a job - 12 m	Same	Same		
DEPB_66C	DEP_66C	DEP_66C	Dep - level of interference - close relationships - 12 m	Same	Same		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
DEPB_66D	DEP_66D	DEP_66D	Dep - level of interference - social life - 12 m	Same	Same		
DEPB_68	DEP_68	DEP_68	Depression - num of days/365 totally unable - 12 m	Same	Same		
DEPB_72	DEP_72	DEP_72	Depression - consulted MD/other professional - life	Same	Same		
DEPB_86	DEP_86	DEP_86	Depression - professional treatment - 12 m	Same	Same		
DEPB_87	DEP_87	DEP_87	Depression - hospitalized overnight - life	Same	Same		
DEPBDA1	DEPDA1	DEPDA1	Depression - Criterion A Part 1 - life - (D)	Same	Same		
DEPBDA21	DEPDA21	DEPDA21	Depression - Criterion A Part 2 - Symptom 1 - life - (D)	Same	Same		
DEPBDA22	DEPDA22	DEPDA22	Depression - Criterion A Part 2 - Symptom 2 - life - (D)	Same	Same		
DEPBDA23	DEPDA23	DEPDA23	Depression - Criterion A Part 2 - Symptom 3 - life - (D)	Same	Same		
DEPBDA24	DEPDA24	DEPDA24	Depression - Criterion A Part 2 - Symptom 4 - life - (D)	Same	Same		
DEPBDA25	DEPDA25	DEPDA25	Depression - Criterion A Part 2 - Symptom 5 - life - (D)	Same	Same		
DEPBDA26	DEPDA26	DEPDA26	Depression - Criterion A Part 2 - Symptom 6 - life - (D)	Same	Same		
DEPBDA27	DEPDA27	DEPDA27	Depression - Criterion A Part 2 - Symptom 7 - life - (D)	Same	Same		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
DEPBDA28	DEPDA28	DEPDA28	Depression - Criterion A Part 2 - Symptom 8 - life - (D)	Same	Same		
DEPBDA29	DEPDA29	DEPDA29	Depression - Criterion A Part 2 - Symptom 9 - life - (D)	Same	Same		
DEPBDA2	DEPDA2	DEPDA2	Depression - Lifetime Criterion A Part 2 - (D)	Same	Same		
DEPBDA	DEPDA	DEPDA	Depression - Criterion A Part 1 and 2 - life - (D)	Same	Same		
DEPBDC1	DEPDC1	DEPDC1	Depression - Lifetime Criterion C Part 1 - (D)	Same	Same		
DEPBD664	DEPD664	DEPD664	Depression - Interference Level (cut-off at 4) - 12 m - (D)	Same	Same		
DEPBDC2	DEPDC2	DEPDC2	Depression - Criterion C Part 2 - life - (D)	Same	Same		
DEPBDC	DEPDC	DEPDC	Depression - Criterion C Part 1 and 2 - life - (D)	Same	Same		
DEPBDE1	DEPDE1	DEPDE1	Depression - Criterion E Part 1 - life - (D)	Same	Same		
DEPBDE21	DEPDE21	DEPDE21	Depression - Criterion E Part 2.1 - life - (D)	Same	Modified		Small technical change, concept unchanged. See DV Specifications.
DEPBDE22	DEPDE22	DEPDE22	Depression - Criterion E Part 2.2 - life - (D)	Same	Modified		Small technical change, concept unchanged. See DV Specifications.
DEPBDE26	DEPDE26	DEPDE26	Depression - Criterion E Part 2.6 - life - (D)	Same	Modified		Small technical change, concept unchanged. See DV Specifications.

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
DEPBDE2	DEPDE2	DEPDE2	Depression - Criterion E Part 2 - life - (D)	Same	Modified		Removed first and 12-month episode duration as input variables.
DEPBD667	DEPD667	DEPD667	Depression - Criterion E Part 3A1 - High interfer -12 m (D)	Same	Same		
DEPBDE3A	DEPDE3A	DEPDE3A	Depression - Criterion E Part 3A - life - (D)	Same	Same		
DEPBDE3B	DEPDE3B	DEPDE3B	Depression - Criterion E Part 3B - life - (D)	Same	Same		
DEPBDE3C	DEPDE3C	DEPDE3C	Depression - Criterion E Part 3C - life - (D)	Same	Same		
DEPBDE3E	DEPDE3E	DEPDE3E	Depression - Criterion E Part 3E - life - (D)	Same	Same		
DEPBDE3	DEPDE3	DEPDE3	Depression - Criterion E Part 3 - life - (D)	Same	Same		
DEPBDE	DEPDE	DEPDE	Depression - Criterion E Part 1 & 2 & 3 - life - (D)	Same	Same		
DEPBDDPS	DEPDDPS	DEPDDPS	Major Depressive Episode - Algorithm - life - (D)	Same	Same		
DEPBD22Y	DEPD22Y	DEPD22Y	Depression - Worst episode - duration - (D)	Same	Modified		Small technical change, concept unchanged. See DV Specifications
DEPBD23Y	DEPD23Y	DEPD23Y	Depression - Most recent episode - duration (D)	Same	Modified		Small technical change, concept unchanged. See DV Specifications
DEPBD39Y	DEPD39Y	DEPD39Y	Depression - Longest episode - duration (D)	Same	Modified		Small technical change, concept unchanged. See DV Specifications.
DEPBDREC	DEPDREC	DEPDREC	Major depressive episode - Most recent episode - age - (D)	Same	Modified		Removed age of onset as input variable.

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
DEPBDPER	DEPDPER	DEPDPER	Major depressive episode - persistence - years - (D)	Same	Modified		Removed first and 12-month episode duration as input variables.
DEPBDYA	DEPDYA	DEPDYA	Depression - Criterion A - 12 m - (D)	Same	Same		
DEPBDYB	DEPDYB	DEPDYB	Depression - Criterion B - 12 m - (D)	Same	Modified		Removed age of onset as input variable.
DEPBDYC	DEPDYC	DEPDYC	Depression - Criterion C - 12 m - (D)	Same	Same		
DEPBDDY	DEPDDY	DEPDDY	Major Depressive Episode - Algorithm - 12 m - (D)	Same	Same		
DEPBFSLT	DEPFSLT	DEPFSLT	Flag: Suicide - thought - life - (F)	Same	Modified		Derived variable conditions changed, concept unchanged. See DV Specifications.
DEPBFSYT	DEPFSYT	DEPFSYT	Flag: Suicide - thought - 12 m - (F)	Same	Modified		Derived variable conditions changed, questionnaire error corrected. See DV Specifications.
	DEPFSLP	DEPFSLP	Flag: Suicide - plan - life - (F)	Same	Added		
	DEPFSYP	DEPFSYP	Flag: Suicide - plan - 12 m - (F)	Same	Added		
DEPBFSLA	DEPFSLA	DEPFSLA	Flag: Suicide - attempt - life - (F)	Same	Modified		Derived variable conditions changed, concept unchanged. See DV Specifications.
DEPBFSYA	DEPFSYA	DEPFSYA	Flag: Suicide - attempt - 12 m - (F)	Same	Modified		Derived variable conditions changed, questionnaire error corrected. See DV Specifications.

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
DEPBFINT	DEPFINT	DEPFINT	Flag: Depression - Interference - 12 m - (F)	Same	Same		
DEPBDINT	DEPDINT	DEPDINT	Depression - Interference - Mean - 12 m - (D)	Same	Same		
MIAB_01	MIA_01	MIA_01	Excited/energetic - changes in thinking/behaviour	Same	Same		
MIAB_03	MIA_03	MIA_02	Excited/energetic - worst episode	Same	Same		
MIAB_03A	MIA_03A	MIA_03A	Excited/energetic - worst episode - age	Same	Same		
MIAB_03B1	MIA_03B1	MIA_03B	Excited/energetic - worst epsiode - duration	Same	Same		
MIAB_03B2	MIA_03B2	MIA_N03B	Excited/energetic - worst episode - unit: D/W/M/Y	Same	Modified		Follow-up questions consolidated for ease of programming
MIAB_03C	MIA_03C	MIA_03C	Excited/energetic - most recent episode - age	Same	Same		
MIAB_03D1	MIA_03D1	MIA_03D	Excited/energetic - most recent episode - duration	Same	Same		
MIAB_03D2	MIA_03D2	MIA_N03D	Excited/energetic - most recent episode - unit: D/W/M/Y	Same	Modified		Follow-up questions consolidated for ease of programming
MIAB_04	MIA_04	MIA_04	Symptom - excited/energetic - irritable/grouchy	Same	Same		
MIAB_05	MIA_05	MIA_05A	Irritable/grouchy - changes in thinking/behaviour	Same	Same		
MIAB_06	MIA_06	MIA_05B	Irritable/grouchy - worst episode - life	Same	Same		
MIAB_06A	MIA_06A	MIA_06A	Irritable/grouchy - worst episode - age	Same	Same		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
MIAB_06B1	MIA_06B1	MIA_06B	Irritable/grouchy - worst episode - duration	Same	Same		
MIAB_06B2	MIA_06B2	MIA_N06B	Irritable/grouchy - worst episode - unit: D/W/M/Y	Same	Modified		Follow-up questions consolidated for ease of programming
MIAB_06C	MIA_06C	MIA_06C	Irritable/grouchy - most recent episode - age	Same	Same		
MIAB_06D1	MIA_06D1	MIA_06D	Irritable/grouchy - most recent episode - duration	Same	Same		
MIAB_06D2	MIA_06D2	MIA_N06D	Irritable/grouchy - most recent episode - unit: D/W/M/Y	Same	Modified		Follow-up questions consolidated for ease of programming
MIAB_07A	MIA_07A	MIA_07A	Symptom - became restless or fidgety	Same	Same		
MIAB_07B	MIA_07B	MIA_07B	Symptom - became overly friendly / outgoing with people	Same	Same		
MIAB_07C	MIA_07C	MIA_07C	Symptom - behaved inappropriately	Same	Same		
MIAB_07D	MIA_07D	MIA_07D	Symptom - was more interested in sex than usual	Same	Same		
MIAB_07E	MIA_07E	MIA_07E	Symptom - tried to do things that were impossible to do	Same	Same		
MIAB_07F	MIA_07F	MIA_07F	Symptom - talked a lot more than usual	Same	Same		
MIAB_07G	MIA_07G	MIA_07G	Symptom - constantly kept changing plans / activities	Same	Same		
MIAB_07H	MIA_07H	MIA_07H	Symptom - was easily distracted	Same	Same		
MIAB_07I	MIA_07I	MIA_07I	Symptom - had thoughts jumping	Same	Same		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
			from one thing to another				
MIAB_07J	MIA_07J	MIA_07J	Symptom - slept far less than usual and was not tired	Same	Same		
MIAB_07K	MIA_07K	MIA_07K	Symptom - was involved in foolish investments for making $	Same	Same		
MIAB_07L	MIA_07L	MIA_07L	Symptom - spent much more money than usual	Same	Same		
MIAB_07M	MIA_07M	MIA_07M	Symptom - did risky things for pleasure	Same	Same		
MIAB_07N	MIA_07N	MIA_07N	Symptom - had a greatly exaggerated self-confidence	Same	Same		
MIAB_07O	MIA_07O	MIA_07O	Symptom - believed was someone else	Same	Same		
MIAB_08	MIA_08	MIA_08	Number of episodes felt excited/irritable - life	Same	Same		
MIAB_09	MIA_09	MIA_09	Interference - work/social life/pers relations - life	Same	Same		
MIAB_09A	MIA_09A	MIA_09_1	Unable carry out daily activities - frequency - life	Same	Same		
MIAB_10A	MIA_10A	MIA_10A	Mania - any episode - physical/med/drug/alc causes	Same	Same		
MIAB_10B	MIA_10B	MIA_10B	Mania - all episodes - physical/med/drug/alc causes	Same	Same		
MIAB_1CA	MIA_1CA	MIA_10CA	Physical exclusion - exhaustion	Same	Same		
MIAB_1CB	MIA_1CB	MIA_10CB	Physical exclusion - hyperventilation	Same	Same		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
MIAB_1CC	MIA_1CC	MIA_10CC	Physical exclusion - hypochondria	Same	Same		
MIAB_1CD	MIA_1CD	MIA_10CD	Physical exclusion - menstrual cycle	Same	Same		
MIAB_1CE	MIA_1CE	MIA_10CE	Physical exclusion - pregnancy/postpartum	Same	Same		
MIAB_1CF	MIA_1CF	MIA_10CF	Physical exclusion - thyroid disease	Same	Same		
MIAB_1CG	MIA_1CG	MIA_10CG	Physical exclusion - cancer	Same	Same		
MIAB_1CH	MIA_1CH	MIA_10CH	Physical exclusion - overweight	Same	Same		
MIAB_1CI	MIA_1CI	MIA_10CI	Physical exclusion - medication (excl illicit drugs)	Same	Same		
MIAB_1CJ	MIA_1CJ	MIA_10CJ	Physical exclusion - illicit drugs	Same	Same		
MIAB_1CK	MIA_1CK	MIA_10CK	Physical exclusion - alcohol	Same	Same		
MIAB_1CL	MIA_1CL	MIA_10CL	Physical exclusion - chemical / serotonin imbalance	Same	Same		
MIAB_1CM	MIA_1CM	MIA_10CM	Physical exclusion - chronic pain	Same	Same		
MIAB_1CN	MIA_1CN	MIA_10CN	Physical exclusion - caffeine	Same	Same		
MIAB_1CO	MIA_1CO	MIA_10CO	Physical exclusion - no specific diagnosis	Same	Same		
MIAB_1CP	MIA_1CP	MIA_10CP	Physical exclusion - other	Same	Same		
MIAB_19	MIA_19	MIA_19A	Mania - episode - 12 m	Same	Same		
MIAB_191	MIA_191	MIA_19A1	Mania - episode - 12 m	Same	Same		
MIAB_19A	MIA_19A	MIA_19A2	Mania - most recent episode during past 12 months	Same	Same		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
MIAB_19B	MIA_19B	MIA_19B	Mania - number of episodes - 12 m	Same	Same		
MIAB_19C	MIA_19C	MIA_19C	Mania - episode - duration - 12 m	Same	Same		
MIAB_19D	MIA_19D	MIA_19D	Mania - most recent episode - age	Same	Same		
MIAB_20	MIA_20	MIA_20A	Mania - num of episodes 1 week or longer - life	Same	Same		
MIAB_201	MIA_201	MIA_21A	Mania - num episodes >= 1 week brought on: stress	Same	Same		
MIAB_202	MIA_202	MIA_21B	Mania - 1 episode >= 1 wk/brought on: stress/out of blue	Same	Same		
MIAB_22A	MIA_22A	MIA_22A	Mania - longest episode - duration	Same	Same		
MIAB_22B	MIA_22B	MIA_N22A	Mania - longest episode - unit:days/weeks/months/years	Same	Modified		Follow-up questions consolidated for ease of programming
MIAB_24	MIA_24	MIA_24	Mania -1 episode life <1 wk - brought on: stress/out of blue	Same	Same		
MIAB_27A	MIA_27A	MIA_27A	Mania - level of interference - home responsibilities - 12 m	Same	Same		
MIAB_271	MIA_271	MIA_27B1	Mania -level of interference -ability to attend school -12 m	Same	Same		
MIAB_272	MIA_272	MIA_27B2	Mania -level of interference -ability to work at a job -12 m	Same	Same		
MIAB_27C	MIA_27C	MIA_27C	Mania - level of interference - close relationships -12 m	Same	Same		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
MIAB_27D	MIA_27D	MIA_27D	Mania - level of interference - social life - 12 m	Same	Same		
MIAB_29	MIA_29	MIA_29	Mania - num of days/365 totally unable - 12 m	Same	Same		
MIAB_33	MIA_33	MIA_33	Mania - consulted MD/other professional - life	Same	Same		
MIAB_47	MIA_47	MIA_47	Mania - professional treatment - 12 m	Same	Same		
MIAB_48	MIA_48	MIA_48	Mania - hospitalized overnight - life	Same	Same		
MIABDA1	MIADA1	MIADA1	Mania - 2002 Criterion A Part 1 - life - (D)	Same	Same		
MIABDA2	MIADA2	MIADA2	Mania - 2002 Criterion A Part 2 - life - (D)	Same	Same		
MIABDA	MIADA	MIADA	Mania - 2002 Criterion A Part 1 and 2 - life - (D)	Same	Same		
MIABDB1	MIADB1	MIADB1	Mania - 2002 Criterion B - Symptom 1 - life - (D)	Same	Same		
MIABDB2	MIADB2	MIADB2	Mania - 2002 Criterion B - Symptom 2 - life - (D)	Same	Same		
MIABDB3	MIADB3	MIADB3	Mania - 2002 Criterion B - Symptom 3 - life - (D)	Same	Same		
MIABDB4	MIADB4	MIADB4	Mania - 2002 Criterion B - Symptom 4 - life - (D)	Same	Same		
MIABDB5	MIADB5	MIADB5	Mania - 2002 Criterion B - Symptom 5 - life - (D)	Same	Same		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
MIABDB6	MIADB6	MIADB6	Mania - 2002 Criterion B - Symptom 6 - life - (D)	Same	Same		
MIABDB7	MIADB7	MIADB7	Mania - 2002 Criterion B - Symptom 7 - life - (D)	Same	Same		
MIABDBI	MIADBI	MIADBI	Mania - Mood irritable but not excited - life - (D)	Modified	Modified	Derived variable conditions changed, concept unchanged. See DV specifications.	Derived Variable conditions changed to account for error in 2002.
MIABDB	MIADB	MIADB	Mania - 2002 Criterion B (Combined sympt 1 - 7) - life - (D)	Same	Same		
MIABD277	MIAD277	MIAD277	Mania - Criterion D Part 1A - Interference - 12 m - (D)	Same	Same		
MIABDD1	MIADD1	MIADD1	Mania - 2002 Criterion D Part 1 - life - (D)	Same	Same		
MIABDD2	MIADD2	MIADD2	Mania - 2002 Criterion D Part 2 - life - (D)	Modified	Same	Derived variable conditions changed, concept unchanged. See DV specifications.	
MIABDD3	MIADD3	MIADD3	Mania - 2002 Criterion D Part 3 - life - (D)	Same	Same		
MIABDD	MIADD	MIADD	Mania - 2002 Criterion D Part 1 & 2 & 3 - life - (D)	Same	Same		
MIABDEPS	MIADEPS	MIADEPS	Mania - 2002 Algorithm - life - (D)	Same	Same		
MIABDREC	MIADREC	MIADREC	Mania - Most recent episode - age (D)	Same	Modified		Removed age of onset as input variable.

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
MIABD3BY	MIAD3BY	MIAD3BY	Mania -Worst episode (excited/energetic) - duration - Y -(D)	Same	Modified		Small technical change, concept unchanged. See DV Specifications.
MIABD3DY	MIAD3DY	MIAD3DY	Mania -Most recent episode (excite/energ) - duration - Y -(D)	Same	Modified		Small technical change, concept unchanged. See DV Specifications.
MIABD6BY	MIAD6BY	MIAD6BY	Mania - Worst episode (irritable/grouchy) - duration - Y (D)	Same	Modified		Small technical change, concept unchanged. See DV Specifications.
MIABD6DY	MIAD6DY	MIAD6DY	Mania - Most recent episode (irrit/grchy) - duration - Y (D)	Same	Modified		Small technical change, concept unchanged. See DV Specifications.
MIABD22Y	MIAD22Y	MIAD22Y	Mania - Longest episode - duration - years - (D)	Same	Modified		Small technical change, concept unchanged. See DV Specifications.
MIABDPER	MIADPER	MIADPER	Mania - Persistence - years - (D)	Same	Modified		Removed first and 12-month episode duration as input variables.
MIABDYA	MIADYA	MIADYA	Mania - 2002 Criterion A - 12 m - (D)	Same	Same		
MIABDYB	MIADYB	MIADYB	Mania - 2002 Criterion B - 12 m - (D)	Same	Modified		Removed age of onset as input variable.
MIABDYC	MIADYC	MIADYC	Mania - 2002 Criterion C - 12 m - (D)	Same	Same		
MIABDEY	MIADEY	MIADEY	Mania - 2002 Algorithm - 12 m - (D)	Same	Same		
MIABDINT	MIADINT	MIADINT	Mania - Interference - Mean - 12 m - (D)	Same	Same		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
MIABFINT	MIAFINT	MIAFINT	Flag: Mania - Interference - 12 m - (F)	Same	Same		
	HYPDA1	HYPDA1	Hypomania - Criterion A Part 1 - life - (D)	Same	Added		
	HYPDA2	HYPDA2	Hypomania - Criterion A Part 2 - life - (D)	Same	Added		
	HYPDA	HYPDA	Hypomania - Criterion A Part 1 and 2 - life - (D)	Same	Added		
	HYPDB	HYPDB	Hypomania - Criterion B - life - (D)	Same	Added		
	HYPD277	HYPD277	Hypomania - Interference - work/social - (D)	Same	Added		
	HYPDC	HYPDC	Hypomania - Criterion C - life - (D)	Same	Added		
	HYPDE1	HYPDE1	Hypomania - Criterion E Part 1 - life - (D)	Same	Added		
	HYPDE2	HYPDE2	Hypomania - Criterion E Part 2 - life - (D)	Same	Added		
	HYPDE3	HYPDE3	Hypomania - Criterion E Part 3 - life - (D)	Same	Added		
	HYPDE	HYPDE	Hypomania - Criterion E Part 1 & 2 & 3 - life - (D)	Same	Added		
	HYPDDUR	HYPDDUR	Hypomania - Longest duration (4-6 days) - (D)	Same	Added		
	HYPDEPS	HYPDEPS	Hypomania - 2002 Algorithm - life - (D)	Same	Added		
	HYPDL	HYPDL	Hypomania - Algorithm - life - (D)	Same	Added		
	HYPDEY	HYPDEY	Hypomania - Algorithm - 12 m - (D)	Same	Added		
	HYPFINT	HYPFINT	Flag: Hypomania - Interference - 12 m - (F)	Same	Added		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
	HYPDINT	HYPDINT	Hypomania - Interference - Mean - 12 m - (D)	Same	Added		
	BIPDA	BIPDA	Bipolar I and II - super-symptoms - sum "yes" (D)	Same	Added		
	BIPDB	BIPDB	Bipolar I and II - super-symptoms - sum "no" (D)	Same	Added		
	BIPD1	BIPD1	Bipolar I - Algorithm - life - (D)	Same	Added		
	BIPD2A	BIPD2A	Bipolar II - Criterion A - life - (D)	Same	Added		
	BIPD2B	BIPD2B	Bipolar II - Criterion B - life - (D)	Same	Added		
	BIPD2C	BIPD2C	Bipolar II - Criterion C - life - (D)	Same	Added		
	BIPD2D1	BIPD2D1	Bipolar II - Criterion D - part 1 - life - (D)	Same	Added		
	BIPD2D2	BIPD2D2	Bipolar II - Criterion D - part 2 - life - (D)	Same	Added		
	BIPD2D3	BIPD2D3	Bipolar II - Criterion D - part 3 - life - (D)	Same	Added		
	BIPD2D	BIPD2D	Bipolar II - Criterion D - life - (D)	Same	Added		
	BIPD2	BIPD2	Bipolar II - Algorithm - life - (D)	Same	Added		
	BIPD1Y	BIPD1Y	Bipolar I - Algorithm - 12 m - (D)	Same	Added		
	BIPD2Y	BIPD2Y	Bipolar II - Algorithm - 12 m - (D)	Same	Added		
	BIPDL	BIPDL	Bipolar Disorder - Algorithm - life - (D)	Same	Added		
	BIPDY	BIPDY	Bipolar Disorder - Algorithm - 12 m - (D)	Same	Added		
	BIPDINT1	BIPDINT1	Bipolar I - Interference - Mean - (D)	Same	Added		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
	BIPDINT2	BIPDINT2	Bipolar II - Interference - Mean - (D)	Same	Added		
	BIPDINT	BIPDINT	Bipolar Disorder - Interference - Mean - (D)	Same	Added		
	BIPFINT1	BIPFINT1	Flag: Bipolar I - Interference - (F)	Same	Added		
	BIPFINT2	BIPFINT2	Flag: Bipolar II - Interference - (F)	Same	Added		
	BIPFINT	BIPFINT	Flag: Bipolar Disorder - Interference - (F)	Same	Added		
	GAD_01A	GAD_01A	Worried - Anxious or worried about everything	Same	Added		
	GAD_01B	GAD_01B	Worried - Anxious or worried about nothing in particular	Same	Added		
	GAD_01C	GAD_01C	Worried - Finances	Same	Added		
	GAD_01D	GAD_01D	Worried - Success at school or work	Same	Added		
	GAD_01E	GAD_01E	Worried - Social life	Same	Added		
	GAD_01F	GAD_01F	Worried - Love life	Same	Added		
	GAD_01G	GAD_01G	Worried - Relationships at school or work	Same	Added		
	GAD_01H	GAD_01H	Worried - Relationships with family	Same	Added		
	GAD_01I	GAD_01I	Worried - Physical appearance	Same	Added		
	GAD_01J	GAD_01J	Worried - Own physical health	Same	Added		
	GAD_01K	GAD_01K	Worried - Own mental health	Same	Added		
	GAD_01L	GAD_01L	Worried - Alcohol or drug use	Same	Added		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
	GAD_01M	GAD_01M	Worried - Being away from home or apart from loved ones	Same	Added		
	GAD_01N	GAD_01N	Worried - The health or welfare of loved ones	Same	Added		
	GAD_01O	GAD_01O	Worried - Social phobias	Same	Added		
	GAD_01P	GAD_01P	Worried - Agoraphobia	Same	Added		
	GAD_01Q	GAD_01Q	Worried - Specific phobias	Same	Added		
	GAD_01R	GAD_01R	Worried - Obsessions	Same	Added		
	GAD_01S	GAD_01S	Worried - Compulsions	Same	Added		
	GAD_01T	GAD_01T	Worried - Crime / violence	Same	Added		
	GAD_01U	GAD_01U	Worried - Economy	Same	Added		
	GAD_01V	GAD_01V	Worried - Environment	Same	Added		
	GAD_01W	GAD_01W	Worried - Moral decline of society	Same	Added		
	GAD_01X	GAD_01X	Worried - War / revolution	Same	Added		
	GAD_01Y	GAD_01Y	Worried - Other	Same	Added		
	GAD_02A	GAD_02A	Had other source of worry - 1	Same	Added		
	GAD_02B	GAD_02B	Had other source of worry - 2	Same	Added		
	GAD_03	GAD_03	Feelings of worry - excessive/unreasonable/a lot stronger	Same	Added		
	GAD_04	GAD_04	Difficult to control anxiety feelings - frequency	Same	Added		
	GAD_04A	GAD_04_1	So nervous could not think about anything else - frequency	Same	Added		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
	GAD_051	GAD_05	Longest period when feeling worried most days - life	Same	Added		
	GAD_05A1	GAD_05_1	Felt anxious most days - Longest period - months/years	Same	Added		
	GAD_05C	GAD_05_3	Had period lasting 6 months or longer when was worried	Same	Added		
	GAD_09A	GAD_09A	Symptom - felt restless/ keyed up/ on edge	Same	Added		
	GAD_09B	GAD_09B	Symptom - got tired easily	Same	Added		
	GAD_09C	GAD_09C	Symptom - was more irritable than usual	Same	Added		
	GAD_09D	GAD_09D	Symptom - difficulty concentrating	Same	Added		
	GAD_09E	GAD_09E	Symptom - had tense sore aching muscles	Same	Added		
	GAD_09F	GAD_09F	Symptom - have trouble falling or staying asleep	Same	Added		
	GAD_10A	GAD_10A	Symptom - had heart pounding or racing	Same	Added		
	GAD_10B	GAD_10B	Symptom - often sweat	Same	Added		
	GAD_10C	GAD_10C	Symptom - had trembles or shakes	Same	Added		
	GAD_10D	GAD_10D	Symptom - had dry mouth	Same	Added		
	GAD_10E	GAD_10E	Symptom - was sad or depressed most of the time	Same	Added		
	GAD_13A	GAD_13A	Symptom - felt dizzy or lightheaded	Same	Added		
	GAD_13B	GAD_13B	Symptom - was short of breath	Same	Added		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
	GAD_13C	GAD_13C	Symptom - felt like choking	Same	Added		
	GAD_13D	GAD_13D	Symptom - had pain/discomfort in chest	Same	Added		
	GAD_13E	GAD_13E	Symptom - had pain/discomfort in stomach	Same	Added		
	GAD_13F	GAD_13F	Symptom - had nausea	Same	Added		
	GAD_13G	GAD_13G	Symptom - felt was unreal	Same	Added		
	GAD_13H	GAD_13H	Symptom - felt things around were unreal	Same	Added		
	GAD_13I	GAD_13I	Symptom - was afraid that might lose control/go crazy	Same	Added		
	GAD_13J	GAD_13J	Symptom - was afraid that might pass out	Same	Added		
	GAD_13K	GAD_13K	Symptom - was afraid that might die	Same	Added		
	GAD_13L	GAD_13L	Symptom - had hot flushes or chills	Same	Added		
	GAD_13M	GAD_13M	Symptom - had numbness or tingling sensations	Same	Added		
	GAD_13N	GAD_13N	Symptom - felt like had a lump in the throat	Same	Added		
	GAD_13O	GAD_13O	Symptom - was easily startled	Same	Added		
	GAD_15	GAD_15	Emotional distress - intensity - life	Same	Added		
	GAD_17	GAD_17	Interference - work/social life/relationships - life	Same	Added		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
	GAD_17A	GAD_17_1	Anxiety/unable to carry out daily act./ frequency - life	Same	Added		
	GAD_18A	GAD_18A	Anxiety - any episode/ physical/med/drugs/alc causes	Same	Added		
	GAD_18B	GAD_18B	Anxiety - all episodes - physical/med/drugs/alc causes	Same	Added		
	GAD_18CA	GAD_18CA	Physical exclusion - exhaustion	Same	Added		
	GAD_18CB	GAD_18CB	Physical exclusion - hyperventilation	Same	Added		
	GAD_18CC	GAD_18CC	Physical exclusion - hypochondria	Same	Added		
	GAD_18CD	GAD_18CD	Physical exclusion - menstrual cycle	Same	Added		
	GAD_18CE	GAD_18CE	Physical exclusion - pregnancy / postpartum	Same	Added		
	GAD_18CF	GAD_18CF	Physical exclusion - thyroid disease	Same	Added		
	GAD_18CG	GAD_18CG	Physical exclusion - cancer	Same	Added		
	GAD_18CH	GAD_18CH	Physical exclusion - overweight	Same	Added		
	GAD_18CI	GAD_18CI	Physical exclusion - medication (excl illicit drugs)	Same	Added		
	GAD_18CJ	GAD_18CJ	Physical exclusion - illicit drugs	Same	Added		
	GAD_18CK	GAD_18CK	Physical exclusion - alcohol	Same	Added		
	GAD_18CL	GAD_18CL	Physical exclusion - chemical / serotonin imbalance	Same	Added		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
	GAD_18CM	GAD_18CM	Physical exclusion - chronic pain	Same	Added		
	GAD_18CN	GAD_18CN	Physical exclusion - caffeine	Same	Added		
	GAD_18CO	GAD_18CO	Physical exclusion - no specific diagnosis	Same	Added		
	GAD_18CP	GAD_18CP	Physical exclusion - other	Same	Added		
	GAD_26D	GAD_26D	Number of episodes lasting 6 months or longer - life	Same	Added		
	GAD_27	GAD_27	Gen anx dis - episode - past 12 months	Same	Added		
	GAD_27A	GAD_27_1	Gen anx dis - most recent episode during past 12 months	Same	Added		
	GAD_27C	GAD_27_2	Gen anx dis - most recent episode - age	Same	Added		
	GAD_311	GAD_31	Gen anx dis - episodes >=6 months - num brought on by stress	Same	Added		
	GAD_26C	GAD_32	Gen anx dis - 1 epi life >= 6 m/brought on: stress/out blue	Same	Added		
	GAD_38A	GAD_38A	Gen anx dis - level interference/home responsibilities - 12m	Same	Added		
	GAD_381	GAD_38B1	Gen anx dis - level interference/ability attend school -12 m	Same	Added		
	GAD_382	GAD_38B2	Gen anx dis - level of interference - ability to work - 12 m	Same	Added		
	GAD_38C	GAD_38C	Gen anx dis - level interference/close relationships - 12m	Same	Added		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
	GAD_38D	GAD_38D	Gen anx dis - level of interference - social life - 12 m	Same	Added		
	GAD_40	GAD_40	Gen anx dis - num days/365 totally unable - 12 m	Same	Added		
	GAD_44	GAD_44	Gen anx dis - consulted MD/other professional - life	Same	Added		
	GAD_58	GAD_58	Gen anx dis - professional treatment - 12 m	Same	Added		
	GAD_59	GAD_59	Gen anx dis - hospitalized overnight - life	Same	Added		
	GADDA1	GADDA1	Gen anx dis - Criterion A Part 1 - life - (D)	Same	Added		
	GADDA2	GADDA2	Gen anx dis - Criterion A Part 2 - life - (D)	Same	Added		
	GADDA	GADDA	Gen anx dis - Criterion A Part 1and 2 - life - (D)	Same	Added		
	GADDB	GADDB	Generalized anxiety disorder - Criterion B - life - (D)	Same	Added		
	GADDC1	GADDC1	Gen anx dis - Criterion C Part 1 - life - (D)	Same	Added		
	GADDC2	GADDC2	Gen anx dis - Criteria C Part 2 - life - (D)	Same	Added		
	GADDC	GADDC	Generalized anxiety disorder - Criterion C - life - (D)	Same	Added		
	GADDD	GADDD	Generalized anxiety disorder - Criterion D - life - (D)	Same	Added		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
	GADDE1	GADDE1	Gen anx dis - Criterion E Part 1 - life - (D)	Same	Added		
	GADD384	GADD384	Gen anx dis - interference level -12 m - (D)	Same	Added		
	GADDE2	GADDE2	Gen anx dis - Criterion E Part 2 - life - (D)	Same	Added		
	GADDE	GADDE	Gen anx dis - Criterion E Part 1 and 2 - life - (D)	Same	Added		
	GADDGDS	GADDGDS	Generalized anxiety disorder - algorithm - life - (D)	Same	Added		
	GADD05Y	GADD05Y	Gen anx dis - Longest episode - duration - (D)	Same	Added		
	GADDREC	GADDREC	Gen anx dis - Most recent episode - age - (D)	Same	Added		
	GADDYA	GADDYA	Gen anx dis - Criterion A - 12 m - (D)	Same	Added		
	GADDYB	GADDYB	Gen anx dis - Criterion B - 12 m - (D)	Same	Added		
	GADDYC	GADDYC	Gen anx dis - Criterion C - 12 m - (D)	Same	Added		
	GADDDY	GADDDY	Generalized anxiety disorder - algorithm - 12 m - (D)	Same	Added		
	GADDINT	GADDINT	Generalized anxiety disorder - Interference - Mean - 12 m (D)	Same	Added		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
	GADFINT	GADFINT	Generalized anxiety disorder - Interference - 12 m - (F)	Same	Added		
SOPB_01A		SOP_01A	Felt shy/afraid - Meeting new people - life	Added			
SOPB_01B		SOP_01B	Felt shy/afraid - Talking to people in authority - life	Added			
SOPB_01C		SOP_01C	Felt shy/afraid - Speaking up in a meeting or class - life	Added			
SOPB_01D		SOP_01D	Felt shy/afraid - Going parties/other social gatherings - life	Added			
SOPB_01E		SOP_01E	Felt shy/afraid - When performing/giving a talk - life	Added			
SOPB_01F		SOP_01F	Felt shy/afraid - Taking imp exam/interviewing for job - life	Added			
SOPB_01G		SOP_01G	Felt shy/afraid - Working while someone watches you - life	Added			
SOPB_01H		SOP_01H	Felt shy/afraid - Entering room when others present - life	Added			
SOPB_01I		SOP_01I	Felt shy/afraid - Talking with people you donâ€™t know well - life	Added			
SOPB_01J		SOP_01J	Felt shy/afraid - Expres disag to	Added			

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
			people donâ€™t know well - life				
SOPB_01K		SOP_01K	Felt shy/afraid - Writing/eat/drink while someone watches - life	Added			
SOPB_01L		SOP_01L	Felt shy/afraid - When using public bathroom away home - life	Added			
SOPB_01M		SOP_01M	Felt shy/afraid - When going on a date - life	Added			
SOPB_01N		SOP_01N	Felt shy/afraid - Other situation where centre attention - life	Added			
SOPB_08A		SOP_08A	Most severe situation you blush/shake - life	Added			
SOPB_08B		SOP_08B	Most severe situation fear lose control of bowels/bladder - life	Added			
SOPB_08C		SOP_08C	Most severe situation fear you might vomit - life	Added			
SOPB_09A		SOP_09A	Faced with situation - Your heart pound or race - life	Added			
SOPB_09B		SOP_09B	Faced with situation - You sweat - life	Added			
SOPB_09C		SOP_09C	Faced with situation - You tremble - life	Added			
SOPB_09D		SOP_09D	Faced with situation - You feel sick to your stomach - life	Added			
SOPB_09E		SOP_09E	Faced with situation - You have a dry mouth - life	Added			

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
SOPB_09F		SOP_09F	Faced with situation - You have hot flushes or chills - life	Added			
SOPB_09G		SOP_09G	Faced with situation - Feel numbness/tingling sensations - life	Added			
SOPB_09H		SOP_09H	Faced with situation - Have trouble breathing normally - life	Added			
SOPB_09I		SOP_09I	Faced with situation - Feel like you were choking - life	Added			
SOPB_09J		SOP_09J	Faced with situation - Have pain/discomfort in chest - life	Added			
SOPB_09K		SOP_09K	Faced with situation - You feel dizzy or faint - life	Added			
SOPB_09L		SOP_09L	Faced with situation - Were you afraid that you might die - life	Added			
SOPB_09M		SOP_09M	Faced with situation - Fear lose control/crazy/pass out - life	Added			
SOPB_09N		SOP_09N	Faced with situation - Feel not really there - life	Added			
SOPB_09O		SOP_09O	Faced with situation - Feel things around you not real - life	Added			
SOPB_10		SOP_10A	Were in situation - Afraid might have a panic attack - life	Added			

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
SOPB_10A		SOP_10B	Were in situation - Have a panic attack - life	Added			
SOPB_11		SOP_11	Afraid that you might be trapped or unable to escape	Added			
SOPB_12		SOP_12A	Were in situation - Afraid do something embar/humiliating - life	Added			
SOPB_12A		SOP_12B	Afraid that you might embarrass other people	Added			
SOPB_13		SOP_13	Afraid people might look/talk/think negative about you	Added			
SOPB_14		SOP_14A	Afraid that you might be the focus of attention	Added			
SOPB_14A		SOP_14B	What was it you feared most about situation life	Added			
SOPB_15		SOP_15A	Fear related to embarrassment about having health problem or condition	Added			
SOPB_15A		SOP_15BA	Emotional or mental health problem or condition	Added			
SOPB_15B		SOP_15BB	Alcohol or drug problem	Added			
SOPB_15C		SOP_15BC	Speech, vision or hearing problem	Added			
SOPB_15D		SOP_15BD	Movement or coordination problem	Added			
SOPB_15E		SOP_15BE	Facial or body disfigurement, weight	Added			

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
			or body image problem				
SOPB_15F		SOP_15BF	Bad odour or sweating	Added			
SOPB_15G		SOP_15BG	Other physical health problem	Added			
SOPB_15H		SOP_15BH	Pregnancy	Added			
SOPB_16		SOP_16	How much did fear or avoidance of situation interfere - life	Added			
SOPB_17		SOP_17	Felt emotionally upset, worried or disappointed with yourself	Added			
SOPB_18		SOP_18A	Last time you either strongly feared or avoided situation	Added			
SOPB_18A		SOP_18B	Age last time you strongly feared or avoided situation	Added			
SOPB_19		SOP_19	Situation today - How strong would your fear be	Added			
SOPB_20		SOP_20	How often did you avoid situation - 12 mo	Added			
SOPB_21A		SOP_21A	How much fear/avoidance of situations interfere with home resp - 12 mo	Added			
SOPB_211		SOP_21B1	How much interfere with your ability to attend school	Added			
SOPB_212		SOP_21B2	How much interfere with your ability to work at a job	Added			

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
SOPB_21C		SOP_21C	How much interfere with ability form/maintain relationship with others	Added			
SOPB_21D		SOP_21D	How much interfere with your social life	Added			
SOPB_23		SOP_23	Totally unable to work/carry out normal activities	Added			
SOPB_25		SOP_25A	Talk to doctor/prof about fear or avoidance - life	Added			
SOPB_38		SOP_38	Receive professional treatment for your fear - 12 mo	Added			
SOPB_39A		SOP_39A	Were you hospitalized overnight for your fear - life	Added			
SOPBDA1		SOPDA1	Social Phobia - life Criterion A, Part 1	Added			
SOPBDA2		SOPDA2	Social Phobia - life Criterion A, Part 2	Added			
SOPBDA		SOPDA	Social Phobia - life Criterion A (Parts 1, 2 Combined)	Added			
SOPBDB		SOPDB	Social Phobia - life Criterion B	Added			
SOPBDC		SOPDC	Social Phobia - life Criterion C	Added			
SOPBDD1		SOPDD1	Social Phobia - life Criterion D, Part 1	Added			
		SOPD09	Social Phobia - Symptoms of anxiety or panic	Added			
SOPBDD2		SOPDD2	Social Phobia - life Criterion D, Part 2	Added			

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
SOPBDD		SOPDD	Social Phobia - life Criterion D (Parts 1, 2 combined)	Added			
		SOPD215	Social and Occupational Interference	Added			
SOPBDE		SOPDE	Social Phobia - life Criterion E	Added			
SOPBBDSP		SOPD02	Social Phobia - life Algorithm	Added		Criteria F is excluded in 2022	
SOPBDREC		SOPDREC	Social Phobia - Recency Age	Added			
SOPBDYA		SOPDYA	Social Phobia - Past 12 Months, Criterion A	Added			
SOPBDYB		SOPDYB	Social Phobia - Past 12 Months, Criterion B	Added			
SOPBDYC		SOPDYC	Social Phobia - Past 12 Months, Criterion C	Added			
SOPBDPY		SOPDPY	Social Phobia - Past 12 Months Algorithm	Added			
SOPBDINT		SOPDINT	Social Phobia Interference - Mean	Added			
SOPBFINT		SOPFINT	Flag for Social Phobia Interference	Added			
	SMK_01A	SMK_201A	Smoked 100 or more cigarettes - life	Same	Added		
	SMK_01B	SMK_201B	Ever smoked whole cigarette	Same	Added		
	SMK_01C	SMK_201C	Age - smoked first whole cigarette	Same	Added		
	SMK_202	SMK_202	Type of smoker	Same	Added		
	SMK_203	SMK_203	Age - started smoking daily (daily smoker)	Same	Added		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
	SMK_204	SMK_204	Number of cigarettes smoked per day (daily smoker)	Same	Added		
	SMK_05B	SMK_205B	Number of cigarettes smoked per day (occasional smoker)	Same	Added		
	SMK_05C	SMK_205C	Number of days - smoked 1 cigarette or more (occ. smoker)	Same	Added		
	SMK_05D	SMK_205D	Ever smoked cigarettes daily	Same	Added		
	SMK_06A	SMK_206A	Stopped smoking - when (was never a daily smoker)	Same	Added		
	SMK_06B	SMK_206B	Stopped smoking - month (never daily smoker)	Same	Added		
	SMK_06C	SMK_206C	Number of years since stopped smoking	Same	Added		
	SMK_207	SMK_207	Age - started smoking daily (former daily smoker)	Same	Added		
	SMK_208	SMK_208	Number of cigarettes smoked per day (former daily smoker)	Same	Added		
	SMK_09A	SMK_209A	Stopped smoking daily - when stopped (former daily smoker)	Same	Added		
	SMK_09B	SMK_209B	Stopped smoking daily - month (former daily smoker)	Same	Added		
	SMK_09C	SMK_209C	Nbr of yrs since stopped smoking daily (former daily smoker)	Same	Added		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
	SMK_10	SMK_210A	Quit smoking completely (former daily smoker)	Same	Added		
	SMK_10A	SMK_210B	Stopped smoking completely - when (former daily smoker)	Same	Added		
	SMK_10B	SMK_210C	Quit smoking completely - month (former daily smoker)	Same	Added		
	SMK_10C	SMK_210D	Quit smoking completely - num of years (former daily smoker)	Same	Added		
	SMKDSTY	SMKDSTY	Type of smoker - (D)	Same	Added		
	SMKDSTP	SMKDSTP	Number of years since stopped smoking completely - (D)	Same	Added		
	SMKDYCS	SMKDYCS	Number of years smoked (current daily smokers) - (D)	Same	Added		
ALCB_1	AUD_01	AUD_01	Drank alcohol in past 12 months	Same	Same		
ALCB_2	AUD_02	AUD_02	Alcohol consumption - frequency - 12 m	Same	Same		
ALCB_3	AUD_03	AUD_03	Having 5 or more drinks - frequency - 12 m	Same	Same		
ALCB_5B	AUD_04	AUD_04	Ever had a drink - life	Same	Same		
	AUD_05	AUD_05	Drank 12 or more drinks - frequency - 12 m	Same	Added		
	AUD_06	AUD_06	Number of drinks per day - 12 m	Same	Added		
	AUD_08	AUD_08	Drank more in earlier year than past 12 months	Same	Added		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
	AUD_10	AUD_10	Alcohol consumption - years when drank most - frequency	Same	Added		
	AUD_11	AUD_11	Drinks per day - years when drank most	Same	Added		
	AUD_13A	AUD_13A	Alcohol abuse - interfere responsibilities - life	Same	Added		
	AUD_13A1	AUD_13A1	Alcohol abuse - problems with people - life	Same	Added		
	AUD_13B	AUD_13B	Alcohol abuse - continued using/problems - life	Same	Added		
	AUD_13C	AUD_13C	Alcohol abuse - could have been hurt - life	Same	Added		
	AUD_13D	AUD_13D	Alcohol abuse - police contact - life	Same	Added		
	AUD_13E	AUD_13E	Alcohol abuse - police contact - num times - life	Same	Added		
	AUD_16	AUD_16	Alcohol abuse - problem recency	Same	Added		
	AUD_17	AUD_17	Alcohol abuse - most recent problem - age	Same	Added		
	AUD_19A	AUD_19A	Alcohol dependence - strong urge to drink - life	Same	Added		
	AUD_19A1	AUD_19A1	Alcohol dependence - need more - life	Same	Added		
	AUD_19B	AUD_19B	Alcohol dependence - withdrawal - life	Same	Added		
	AUD_19C	AUD_19C	Alcohol dependence - use to stop withdrawal - life	Same	Added		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
	AUD_19D	AUD_19D	Alcohol dependence - use more than intended - life	Same	Added		
	AUD_19E	AUD_19E	Alcohol dependence - use more frequently than intended -life	Same	Added		
	AUD_19F	AUD_19F	Alcohol dependence - unwanted intoxication - life	Same	Added		
	AUD_19G	AUD_19G	Alcohol dependence - unable to cut down usage - life	Same	Added		
	AUD_19H	AUD_19H	Alcohol dependence - spent days drinking/recovering - life	Same	Added		
	AUD_19I	AUD_19I	Alcohol dependence - reduced activities - life	Same	Added		
	AUD_19J	AUD_19J	Alcohol dependence - drank despite serious problem - life	Same	Added		
	AUD_23	AUD_23	Alcohol dependence - 3+ symptoms in same 12 m - life	Same	Added		
	AUD_25	AUD_25	Alcohol dependence - problem recency	Same	Added		
	AUD_26	AUD_26	Alcohol dependence - most recent problem - age	Same	Added		
	AUD_29	AUD_29	Alcohol dependance - num times tried to quit	Same	Added		
ALDB_15A	AUD_35A	AUD_35A	Alcohol- level interference - home responsibilities - 12 m	Same	Modified		Question in 2012 survey refers to period of time that lasted one month or longer in the past 12

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
							months when the respondent drank the most.
ALDB_5B1	AUD_351	AUD_35B1	Alcohol - level interference/ability attend school - 12 m	Same	Modified		Question in 2012 survey refers to period of time that lasted one month or longer in the past 12 months when the respondent drank the most.
ALDB_5B2	AUD_352	AUD_35B2	Alcohol - level of interference/ability to work - 12 m	Same	Modified		Question in 2012 survey refers to period of time that lasted one month or longer in the past 12 months when the respondent drank the most.
ALDB_15C	AUD_35C	AUD_35C	Alcohol - level of interference - close relationships - 12 m	Same	Modified		Question in 2012 survey refers to period of time that lasted one month or longer in the past 12 months when the respondent drank the most.
AUDB_15D	AUD_35D	AUD_35D	Alcohol - level of interference - social life - 12 m	Same	Modified		Question in 2012 survey refers to period of time that lasted one month or longer in the past 12 months when the respondent drank the most.
	AUD_36A	AUD_36A	Alcohol - num of days/365 totally unable - 12 m	Same	Added		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
	AUD_39	AUD_39	Alcohol - consulted MD/other professional - life	Same	Added		
	AUD_41	AUD_41	Alcohol -received professional treatment - 12 m	Same	Added		
	AUD_42	AUD_42	Alcohol - hospitalized overnight - life	Same	Added		
	AUDDTTM	AUDDTTM	Alcohol - Type of drinker - 12 m - (D)	Same	Added		
	AUDFWDY	AUDFWDY	Flag: Ever drank more than in past 12 months (F)	Same	Added		
	AUDDWDY	AUDDWDY	Alcohol - Type of drinker - year most frequent drinking (D)	Same	Added		
	AUDDDA	AUDDDA	Alcohol dependence - Criterion A - life - (D)	Same	Added		
	AUDDDB	AUDDDB	Alcohol dependence - Criterion B - life - (D)	Same	Added		
	AUDDLD	AUDDLD	Alcohol dependence - Algorithm - life - (D)	Same	Added		
	AUDDYD	AUDDYD	Alcohol dependence - Algorithm - 12 m - (D)	Same	Added		
	AUDDAA	AUDDAA	Alcohol abuse - Criterion A - life - (D)	Same	Added		
	AUDDAB	AUDDAB	Alcohol abuse - Criterion B - life - (D)	Same	Added		
	AUDDLA	AUDDLA	Alcohol abuse - Algorithm - life - (D)	Same	Added		
	AUDDYA	AUDDYA	Alcohol abuse - Algorithm - 12 m - (D)	Same	Added		
	AUDDL	AUDDL	Alcohol abuse or dependence - Algorithm - life - (D)	Same	Added		
	AUDDY	AUDDY	Alcohol abuse or dependence - Algorithm - 12 m - (D)	Same	Added		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
ALDBFINT	AUDFINT	AUDFINT	Flag: Alcohol - Interference - 12 m - (F)	Same	Modified		Conditions changed in DV Specifications to account for new questions and disorder criteria.
ALDBDINT	AUDDINT	AUDDINT	Alcohol - Interference - Mean - 12 m - (D)	Same	Modified		Conditions changed in DV Specifications to account for new questions and disorder criteria.
	SUD_01A	SUD_01A	Sedative - used nonmedically - life	Same	Added		
	SUD_01B	SUD_01B	Sedative - used prescribed - life	Same	Added		
	SUD_01C	SUD_01C	Sedative - prescribed - could not stop using	Same	Added		
	SUD_01D	SUD_01D	Sedative - used nonmedically - 12 m	Same	Added		
	SUD_01E	SUD_01E	Sedative - frequency - 12 m	Same	Added		
	SUD_02A	SUD_02A	Stimulant - used nonmedically - life	Same	Added		
	SUD_02B	SUD_02B	Stimulant - used prescribed - life	Same	Added		
	SUD_02C	SUD_02C	Stimulant - prescribed - could not stop using	Same	Added		
	SUD_02D	SUD_02D	Stimulant - used nonmedically - 12 m	Same	Added		
	SUD_02E	SUD_02E	Stimulant - frequency - 12 m	Same	Added		
	SUD_03A	SUD_03A	Analgesic - used nonmedically - life	Same	Added		
	SUD_03B	SUD_03B	Analgesic - used prescribed - life	Same	Added		
	SUD_03C	SUD_03C	Analgesic - prescribed - could not stop using	Same	Added		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
	SUD_03D	SUD_03D	Analgesic - used nonmedically - 12 m	Same	Added		
	SUD_03E	SUD_03E	Analgesic - frequency - 12 m	Same	Added		
IDGB_01	SUD_04A	SUD_04A	Cannabis - used - life	Same	Same		
IDGB_02	SUD_04B	SUD_04B	Cannabis - used - 12 m	Same	Same		
IDGB_03	SUD_04C	SUD_04C	Cannabis - frequency - 12 m	Same	Same		
	SUD_04D	SUD_04D	Cannabis - used more in the past	Same	Added		
	SUD_04E	SUD_04E	Cannabis - year used most often - frequency	Same	Added		
	SUD_04F	SUD_04F	Cannabis - num times used - life	Same	Added		
	SUD_04G	SUD_04G	Cannabis - used more than 50 times - life	Same	Added		
IDGB_04	SUD_05A	SUD_05A	Cocaine or crack - used - life	Same	Modified		Question reworded to match CIDI drug list
IDGB_05	SUD_05B	SUD_05B	Cocaine or crack - used - 12 m	Same	Modified		Question reworded to match CIDI drug list
IDGB_06	SUD_05C	SUD_05C	Cocaine or crack - frequency - 12 m	Same	Modified		Question reworded to match CIDI drug list
IDGB_10	SUD_06A	SUD_06A	Ecstasy (MDMA) - used - life	Same	Modified		Question reworded to match CIDI drug list
IDGB_11	SUD_06B	SUD_06B	Ecstasy (MDMA) - used - 12 m	Same	Modified		Question reworded to match CIDI drug list
IDGB_12	SUD_06C	SUD_06C	Ecstacy (MDMA) - frequency - 12 m	Same	Modified		Question reworded to match CIDI drug list
IDGB_13	SUD_07A	SUD_07A	Hallucinogens/ PCP/ LSD - used - life	Same	Modified		Question reworded to match CIDI drug list
IDGB_14	SUD_07B	SUD_07B	Hallucinogens/ PCP/ LSD - used - 12 m	Same	Modified		Question reworded to match CIDI drug list
IDGB_15	SUD_07C	SUD_07C	Hallucinogens/ PCP/ LSD - frequency - 12 m	Same	Modified		Question reworded to match CIDI drug list

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
IDGB_19	SUD_08A	SUD_08A	Heroin - used - life	Same	Modified		Question reworded to match CIDI drug list
IDGB_20	SUD_08B	SUD_08B	Heroin - used - 12 m	Same	Modified		Question reworded to match CIDI drug list
IDGB_21	SUD_08C	SUD_08C	Heroin - frequency - 12 m	Same	Modified		Question reworded to match CIDI drug list
IDGB_16	SUD_09A	SUD_09A	Inhalants or solvents - used - life	Same	Modified		Question reworded to match CIDI drug list
IDGB_17	SUD_09B	SUD_09B	Inhalants or solvents - used - 12 m	Same	Modified		Question reworded to match CIDI drug list
IDGB_18	SUD_09C	SUD_09C	Inhalants or solvents - frequency - 12 m	Same	Modified		Question reworded to match CIDI drug list
	SUD_10A	SUD_10A	Other illegal drug - used - life	Same	Added		
	SUD_10B	SUD_10B	Other illegal drugs - used - 12 m	Same	Added		
	SUD_10C	SUD_10C	Other illegal drug - frequency - 12 m	Same	Added		
	SUD_21	SUD_21	Cannabis abuse - interfere responsibilities - life	Same	Added		
	SUD_22	SUD_22	Cannabis abuse - problems with people - life	Same	Added		
	SUD_23	SUD_23	Cannabis abuse - continued using/problems - life	Same	Added		
	SUD_24	SUD_24	Cannabis abuse - could have been hurt - life	Same	Added		
	SUD_25	SUD_25	Cannabis abuse - police contact - life	Same	Added		
	SUD_28	SUD_28	Cannabis abuse - problem recency	Same	Added		
	SUD_29	SUD_29	Cannabis abuse - most recent problem - age	Same	Added		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
	SUD_31	SUD_31	Cannabis dependence - strong urge to use - life	Same	Added		
	SUD_32	SUD_32	Cannabis dependence - need more - life	Same	Added		
	SUD_33	SUD_33	Cannabis dependence - withdrawal - life	Same	Added		
	SUD_34	SUD_34	Cannabis dependence - use to stop withdrawal - life	Same	Added		
	SUD_35	SUD_35	Cannabis dependence - use more than intended - life	Same	Added		
	SUD_36	SUD_36	Cannabis dependence - use more frequently then intended-life	Same	Added		
	SUD_37	SUD_37	Cannabis dependence - unable to cut down usage - life	Same	Added		
	SUD_38	SUD_38	Cannabis dependence - spent days using/recovering - life	Same	Added		
	SUD_39	SUD_39	Cannabis dependence - reduced activities - life	Same	Added		
	SUD_40	SUD_40	Cannabis dependence - used despite serious problem - life	Same	Added		
	SUD_43	SUD_43	Cannabis dependence - 3+	Same	Added		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
			symptoms in same 12 m - life				
	SUD_45	SUD_45	Cannabis dependence - problem recency	Same	Added		
	SUD_46	SUD_46	Cannabis dependence - most recent problem - age	Same	Added		
	SUD_51	SUD_51	Drug abuse excl cann - interfere responsibilities - life	Same	Added		
	SUD_52	SUD_52	Drug abuse excl cann - problems with people - life	Same	Added		
	SUD_53	SUD_53	Drug abuse excl cann - continued using/problem - life	Same	Added		
	SUD_54	SUD_54	Drug abuse excl cann - could have be hurt - life	Same	Added		
	SUD_55	SUD_55	Drug abuse excl cann - police contact - life	Same	Added		
	SUD_58	SUD_58	Drug abuse excl cann - problem recency	Same	Added		
	SUD_59	SUD_59	Drug abuse excl cann - most recent problem - age	Same	Added		
	SUD_61	SUD_61	Drug depend excl cann - strong urge to use - life	Same	Added		
	SUD_62	SUD_62	Drug depend excl cann - need more - life	Same	Added		
	SUD_63	SUD_63	Drug depend excl cann - withdrawal - life	Same	Added		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
	SUD_64	SUD_64	Drug depend excl cann - use to stop withdrawal - life	Same	Added		
	SUD_65	SUD_65	Drug depend excl cann - use more than intended - life	Same	Added		
	SUD_66	SUD_66	Drug depend excl cann -use more freq than intended-life	Same	Added		
	SUD_67	SUD_67	Drug depend excl cann - unable to cut down usage - life	Same	Added		
	SUD_68	SUD_68	Drug depend excl cann -spent days using/recovering-life	Same	Added		
	SUD_69	SUD_69	Drug depend excl cann - reduced activities - life	Same	Added		
	SUD_70	SUD_70	Drug depend excl cann-used despite serious problem-life	Same	Added		
	SUD_73	SUD_73	Drug depend excl cann - 3+ symptoms in same 12 m - life	Same	Added		
	SUD_75	SUD_75	Drug depend excl cann - problem recency	Same	Added		
	SUD_76	SUD_76	Drug depend excl cann - most recent problem - age	Same	Added		
	SUD_81A	SUD_81A	Drugs - level interference/home responsibilities - 12 m	Same	Added		
	SUD_811	SUD_81B1	Drugs - level interference/ability attend school - 12 m	Same	Added		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
	SUD_812	SUD_81B2	Drugs - level of interference/ability to work - 12 m	Same	Added		
	SUD_81C	SUD_81C	Drugs - level of interference/close relationships -12 m	Same	Added		
	SUD_81D	SUD_81D	Drugs - level of interference - social life - 12 m	Same	Added		
	SUD_83	SUD_83	Drugs - num of days/365 totally unable - 12 m	Same	Added		
	SUD_87	SUD_87	Drugs - Help received - Talked to a professional - life	Same	Added		
	SUD_89	SUD_89	Drugs - Help received - Professional treatment - 12 mo	Same	Added		
	SUD_90	SUD_90	Drugs - Help received - Been hospitalized overnight - life	Same	Added		
	SUDFLTU	SUDFLTU	Sedative - used nonmedically - life - (F)	Same	Added		
	SUDFLSU	SUDFLSU	Stimulant - used nonmedically - life - (F)	Same	Added		
	SUDFLAU	SUDFLAU	Analgesics - nonmedical use - life - (F)	Same	Added		
IDGBFLCA	SUDFLCA	SUDFLCA	Cannabis - used (incl one time) - life - (F)	Same	Same		
IDGBFLCM	SUDFLCM	SUDFLCM	Cannabis- used (excl one time) - life - (F)	Same	Same		
IDGBFYCM	SUDFYCM	SUDFYCM	Cannabis - used (excl one time) - 12 m - (F)	Same	Same		
IDGBFLCO	SUDFLCO	SUDFLCO	Cocaine/crack - used - life - (F)	Same	Modified		Input questions reworded

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
IDGBFLEX	SUDFLEX	SUDFLEX	MDMA (ecstasy) - used - life - (F)	Same	Modified		Input questions reworded
IDGBFLEX	SUDFLHA	SUDFLHA	Hallucinogens/ PCP/ LSD - used - life - (F)	Same	Modified		Input questions reworded
IDGBFLHE	SUDFLHE	SUDFLHE	Heroin - used - life - (F)	Same	Modified		Input questions reworded
IDGBFLGL	SUDFLGL	SUDFLGL	Inhalant or solvent - used - life - (F)	Same	Modified		Input questions reworded
	SUDFLAO	SUDFLAO	Any other drug - used - life - (F)	Same	Added		
	SUDDLAI	SUDDLAI	Any drug use (incl one time cann) - life - (D)	Same	Added		
	SUDDLAE	SUDDLAE	Any drug use (excl one time cann) - life (D)	Same	Added		
	SUDDLID	SUDDLID	Any drug use (excl cann use) - life - (D)	Same	Added		
	SUDDYAI	SUDDYAI	Any drug use (incl one time cann) - 12 m (D)	Same	Added		
	SUDDYAE	SUDDYAE	Any drug use (excl one time cann) - 12 m (D)	Same	Added		
	SUDDYID	SUDDYID	Any drug use (excl cann use) - 12 m (D)	Same	Added		
	SUDDCDA	SUDDCDA	Cannabis dependence - Criterion A - life - (D)	Same	Added		
	SUDDCDB	SUDDCDB	Cannabis dependence - Criterion B - life - (D)	Same	Added		
	SUDDLCD	SUDDLCD	Cannabis dependence - Algorithm - life - (D)	Same	Added		
	SUDDYCD	SUDDYCD	Cannabis dependence - Algorithm - 12 m - (D)	Same	Added		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
	SUDDODA	SUDDODA	Drug dependence (excl cann) - Criterion A - life - (D)	Same	Added		
	SUDDODB	SUDDODB	Drug dependence (excl cann) - Criterion B - life - (D)	Same	Added		
	SUDDLOD	SUDDLOD	Drug dependence (excl cann) - Algorithm - life - (D)	Same	Added		
	SUDDYOD	SUDDYOD	Drug dependence (excl cann) - Algorithm - 12 m - (D)	Same	Added		
	SUDDCAA	SUDDCAA	Cannabis abuse - Criterion A - life - (D)	Same	Added		
	SUDDCAB	SUDDCAB	Cannabis abuse - Criterion B - life - (D)	Same	Added		
	SUDDLCA	SUDDLCA	Cannabis abuse - Algorithm - life - (D)	Same	Added		
	SUDDYCA	SUDDYCA	Cannabis abuse - Algorithm - 12 m - (D)	Same	Added		
	SUDDOAA	SUDDOAA	Drug abuse (excl cann) - Criterion A - life - (D)	Same	Added		
	SUDDOAB	SUDDOAB	Drug abuse (excl cann) - Criterion B - life - (D)	Same	Added		
	SUDDLOA	SUDDLOA	Drug abuse (excl cann) - Algorithm - life - (D)	Same	Added		
	SUDDYOA	SUDDYOA	Drug abuse (excl cann) - Algorithm - 12 m - (D)	Same	Added		
	SUDDLC	SUDDLC	Cannabis abuse or dependence - life - (D)	Same	Added		
	SUDDYC	SUDDYC	Cannabis abuse or dependence - 12 m - (D)	Same	Added		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
	SUDDLO	SUDDLO	Drug abuse or dependence (excl cann) - life - (D)	Same	Added		
	SUDDYO	SUDDYO	Drug abuse or dependence (excl cann) - 12 m - (D)	Same	Added		
	SUDDL	SUDDL	Drug abuse or dependence (including cann) - life - (D)	Same	Added		
	SUDDY	SUDDY	Drug abuse or dependence (including cann) - 12 m - (D)	Same	Added		
IDGBDINT	SUDFINT	SUDFINT	Substance use - Interference - 12 m - (F)	Same	Modified		Conditions changed in DV Specifications to account for new questions and disorder criteria.
IDGBFINT	SUDDINT	SUDDINT	Substance use - Interference - Mean - 12 m (D)	Same	Modified		Conditions changed in DV Specifications to account for new questions and disorder criteria.
MHPBFL	MHPFL	MHPFL	Any selected disorder (mental or substance) - life (F)	Modified	Modified	Derived variable conditions changed, concept unchanged. See DV specifications.	Selected disorders are different between cycles
MHPBFLM	MHPFLM	MHPFLM	Any mood disorder - life - (F)	Same	Modified		Selected disorders are different between cycles
		MHPFLA	Any anxiety disorder - life	Added			
	MHPFLSA	MHPFLSA	Any substance use disorder (alcohol/drug) - life - (F)	Same	Added		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
MHPBFY	MHPFY	MHPFY	Any selected disorder (mental/substance) - 12 m - (F)	Modified	Modified	Derived variable conditions changed, concept unchanged. See DV specifications.	Selected disorders are different between cycles
MHPBFYM	MHPFYM	MHPFYM	Any mood disorder - 12 m - (F)	Same	Modified		Selected disorders are different between cycles
		MHPFYA	Any anxiety disorder - 12 m	Added			
MHPBFYSA	MHPFYSA	MHPFYSA	Any substance use disorder (alcohol/drug) - 12m - (F)	Same	Modified		Selected disorders are different between cycles
MHPBFINT	MHPFINT	MHPFINT	Any selected disorder (mental/subst) - Interfere -12 m (F)	Modified	Modified	Derived variable conditions changed, concept unchanged. See DV specifications.	Selected disorders are different between cycles
	DAS_01	DAS_01	Difficulty - standing for long periods - 30 d	Same	Added		
	DAS_02	DAS_02	Difficulty - household responsibilities - 30 d	Same	Added		
	DAS_03	DAS_03	Difficulty - learning a new task - 30 d	Same	Added		
	DAS_04	DAS_04	Difficulty - joining in community activities - 30 d	Same	Added		
	DAS_05	DAS_05	Emotionally affected by health problems - 30 d	Same	Added		
	DAS_06	DAS_06	Difficulty - concentrating - 30 d	Same	Added		
	DAS_07	DAS_07	Difficulty - walking long distance - 30 d	Same	Added		
	DAS_08	DAS_08	Difficulty - washing your whole body - 30 d	Same	Added		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
	DAS_09	DAS_09	Difficulty - getting dressed - 30 d	Same	Added		
	DAS_10	DAS_10	Difficulty - dealing with people you do not know - 30 d	Same	Added		
	DAS_11	DAS_11	Difficulty - maintaining a friendship - 30 d	Same	Added		
	DAS_12	DAS_12	Difficulty - day to day work/school - 30 d	Same	Added		
	DAS_13	DAS_13	Interference - difficulties - 30 days	Same	Added		
	DASDSCR	DASDSCR	WHO Disability Assessment Schedule - score - (D)	Same	Added		
SERB_02	SR1_001	SR1_001	Hospitalized for mental health/alcohol/drug - 12 m	Same	Modified		Question reworded and reorded. Can be compared to SERBFHYR in 2002.
	SR1_002	SR1_002	Hospitalized for mental health/alc/drug - num of times -12 m	Same	Added		
	SR1_003	SR1_003	Total time spent in hospital due to mental health - 12 mo	Same	Added		
	SR1_004A	SR1_004A	Consulted for mental health/alc/drugs - psychiatrist - 12 m	Same	Added		
	SR1_004B	SR1_004B	Consulted for ment hlth/alc/drugs - family doctor/GP - 12 m	Same	Added		
	SR1_004C	SR1_004C	Consulted for mental health/alc/drugs - psychologist - 12 m	Same	Added		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
	SR1_004D	SR1_004D	Consulted for ment hlth/alc/drugs - nurse - 12 m	Same	Added		
	SR1_004E	SR1_004E	Consulted social worker or counsellor - 12 months	Same	Added		
	SR1_004F	SR1_004F	Consulted for ment hlth/alc/drugs - family member - 12 m	Same	Added		
	SR1_004G	SR1_004G	Consulted for ment hlth/alc/drugs - friend - 12 m	Same	Added		
	SR1_004H	SR1_004H	Consulted for ment hlth/alc/drugs - co-worker/ boss - 12 m	Same	Added		
	SR1_004I	SR1_004H	Consulted for ment hlth/alc/drugs - teacher/principal - 12 m	Same	Added		
	SR1_004J	SR1_004I	Consulted for ment hlth/alc/drugs - other - 12 m	Same	Added		
	SR1_004K	SR1_004J	Consulted for ment hlth/alc/drugs - none - 12 m	Same	Added		
		SR1_10AA	Talked with psychiatrist - in person	Added			
		SR1_10AB	Talked with psychiatrist - over the phone	Added			
		SR1_10AC	Talked with psychiatrist - using video	Added			
		SR1_10AD	Talked with psychiatrist - text message	Added			

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
SERB_23	SR1_010	SR1_010B	Number of times consulted psychiatrist - 12 mo	Same	Same		
	SR1_011	SR1_011	Consultation average time in minutes - psychiatrist	Same	Added		
SERB_26	SR1_012	SR1_012	Perceived level of help received - psychiatrist	Same	Same		
SERB_27	SR1_013	SR1_013	Stopped seeing this professional - psychiatrist	Same	Same		
SERB_28A	SR1_014A	SR1_014A	Reason stopped - psychia - felt better	Same	Same		
SERB_28B	SR1_014B	SR1_014B	Reason stopped - psychia - completed recommended treatment	Same	Same		
SERB_28C	SR1_014C	SR1_014C	Reason stopped - psychia - thought it was not helping	Same	Same		
SERB_28D	SR1_014D	SR1_014D	Reason stopped - psychia - problem get better without help	Same	Same		
SERB_28E	SR1_014E	SR1_014E	Reason stopped - psychia - couldn't afford to pay	Same	Same		
SERB_28F	SR1_014F	SR1_014F	Reason stopped - psychia - too embarrass to see professional	Same	Same		
SERB_28G	SR1_014G	SR1_014G	Reason stopped - psychia - solve the problem without help	Same	Same		
SERB_28H	SR1_014H	SR1_014H	Reason stopped - psychia - prob like transport/ childcare	Same	Same		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
SERB_28I	SR1_014I	SR1_014I	Reason stopped - psychia - service/prog no longer available	Same	Same		
SERB_28J	SR1_014J	SR1_014J	Reason stopped - psychia - not comfortable w prof approach	Same	Same		
	SR1_014K	SR1_014K	Reason stopped - psychia - discrimination/unfair treatment	Same	Added		
SERB_28K	SR1_014L	SR1_014L	Reason stopped - psychia - other reason	Same	Same		
		SR1_20AA	Talked with family doctor - in person	Added			
		SR1_20AB	Talked with family doctor - over the phone	Added			
		SR1_20AC	Talked with family doctor - using video	Added			
		SR1_20AD	Talked with family doctor - text message	Added			
SERB_33	SR1_020	SR1_020B	Number of times consulted family doctor - 12 mo	Same	Same		
	SR1_021	SR1_021	Consultation/av time in minutes - family dr/gen practitioner	Same	Added		
SERB_36	SR1_022	SR1_022	Perceived level of help received from prof. - family doctor	Same	Same		
SERB_37	SR1_023	SR1_023	Stopped seeing prof. - family doctor	Same	Same		
SERB_38A	SR1_024A	SR1_024A	Reason stopped - dr - felt better	Same	Same		
SERB_38B	SR1_024B	SR1_024B	Reason stopped - dr - completed the	Same	Same		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
			recommended treatment				
SERB_38C	SR1_024C	SR1_024C	Reason stopped - dr - thought it was not helping	Same	Same		
SERB_38D	SR1_024D	SR1_024D	Reason stopped - dr - problem would get better without help	Same	Same		
SERB_38E	SR1_024E	SR1_024E	Reason stopped - dr - couldn't afford to pay	Same	Same		
SERB_38F	SR1_024F	SR1_024F	Reason stopped - dr - too embarrassed to see professional	Same	Same		
SERB_38F	SR1_024G	SR1_024G	Reason stopped - dr - solve the problem without help	Same	Same		
SERB_38H	SR1_024H	SR1_024H	Reason stopped - dr - probs like transportation/ childcare	Same	Same		
SERB_38I	SR1_024I	SR1_024I	Reason stopped - dr - service / program no longer available	Same	Same		
SERB_38J	SR1_024J	SR1_024J	Reason stopped - dr - not comfortable with prof. approach	Same	Same		
	SR1_024K	SR1_024K	Reason stopped - dr - discrimination or unfair treatment	Same	Added		
SERB_38K	SR1_024L	SR1_024L	Reason stopped - dr - other reason	Same	Same		
		SR1_30AA	Talked with psychologist - in person	Added			
		SR1_30AB	Talked with psychologist - over the phone	Added			

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
		SR1_30AC	Talked with psychologist - using video	Added			
		SR1_30AD	Talked with psychologist - text message	Added			
SERB_53	SR1_030	SR1_030B	Number of times consulted psychologist - 12 mo	Same	Same		
	SR1_031	SR1_031	Consultation/avg time in minutes - psychologist	Same	Added		
SERB_56	SR1_032	SR1_032	Perceived level of help received - psychologist	Same	Same		
SERB_57	SR1_033	SR1_033	Stopped seeing this professional - psychologist	Same	Same		
SERB_58A	SR1_034A	SR1_034A	Reason stopped - psychol - felt better	Same	Same		
SERB_58B	SR1_034B	SR1_034B	Reason stopped - psychol - completed recommended treatment	Same	Same		
SERB_58C	SR1_034C	SR1_034C	Reason stopped - psychol - thought it was not helping	Same	Same		
SERB_58D	SR1_034D	SR1_034D	Reason stopped - psychol - problem would get better	Same	Same		
SERB_58E	SR1_034E	SR1_034E	Reason stopped - psychol - couldn't afford to pay	Same	Same		
SERB_58F	SR1_034F	SR1_034F	Reason stopped - psychol - embarrassed to see professional	Same	Same		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
SERB_58G	SR1_034G	SR1_034G	Reason stopped - psychol - solve the problem without help	Same	Same		
SERB_58H	SR1_034H	SR1_034H	Reason stopped - psychol - probs - transportation/ childcare	Same	Same		
SERB_58I	SR1_034I	SR1_034I	Reason stopped - psychol - service/prog no longer available	Same	Same		
SERB_58J	SR1_034J	SR1_034J	Reason stopped - psychol - not comfort. with prof. approach	Same	Same		
	SR1_034K	SR1_034K	Reason stopped - psychol - discrimination/unfair treatment	Same	Added		
SERB_58K	SR1_034L	SR1_034L	Reason stopped - psychol - other reason	Same	Same		
		SR1_40AA	Talked with nurse - in person	Added			
		SR1_40AB	Talked with nurse - over the phone	Added			
		SR1_40AC	Talked with nurse - using video	Added			
		SR1_40AD	Talked with nurse - text message	Added			
SERB_63	SR1_040	SR1_040B	Number of times consulted nurse - 12 mo	Same	Same		
	SR1_041	SR1_041	Consultation/time in average minutes - nurse	Same	Added		
SERB_66	SR1_042	SR1_042	Perceived level of help received from prof. - nurse	Same	Same		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
SERB_67	SR1_043	SR1_043	Stopped seeing this professional - nurse	Same	Same		
SERB_68A	SR1_044A	SR1_044A	Reason stopped - nurse - felt better	Same	Same		
SERB_68B	SR1_044B	SR1_044B	Reason stopped - nurse - completed the recommended treatment	Same	Same		
SERB_68C	SR1_044C	SR1_044C	Reason stopped - nurse - thought it was not helping	Same	Same		
SERB_68D	SR1_044D	SR1_044D	Reason stopped - nurse - prob would get better without help	Same	Same		
SERB_68E	SR1_044E	SR1_044E	Reason stopped - nurse - couldn't afford to pay	Same	Same		
SERB_68F	SR1_044F	SR1_044F	Reason stopped - nurse - too embarrassed to see professional	Same	Same		
SERB_68G	SR1_044G	SR1_044G	Reason stopped - nurse - solve the problem without prof help	Same	Same		
SERB_68K	SR1_044H	SR1_044H	Reason stopped - nurse - prob like transportation/ childcare	Same	Same		
SERB_68I	SR1_044I	SR1_044I	Reason stopped - nurse - service/program no longer available	Same	Same		
SERB_68J	SR1_044J	SR1_044J	Reason stopped - nurse - not comfortable with prof. approach	Same	Same		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
	SR1_044K	SR1_044K	Reason stopped - nurse - discrimination or unfair treatment	Same	Added		
SERB_68K	SR1_044L	SR1_044L	Reason stopped - nurse - other reason	Same	Same		
		SR1_50AA	Talked with social worker - in person	Added			
		SR1_50AB	Talked with social worker - over the phone	Added			
		SR1_50AC	Talked with social worker - using video	Added			
		SR1_50AD	Talked with social worker - text message	Added			
SERB_73	SR1_050	SR1_050B	Number of times consulted social worker - 12 mo	Same	Same		
	SR1_051	SR1_051	Consultation/time in average minutes - social worker	Same	Added		
SERB_76	SR1_052	SR1_052	Perceived level of help received from prof. - social worker	Same	Same		
SERB_77	SR1_053	SR1_053	Stopped seeing this professional/social worker	Same	Same		
SERB_78A	SR1_054A	SR1_054A	Reason stopped - sw - felt better	Same	Same		
SERB_78B	SR1_054B	SR1_054B	Reason stopped - sw - completed the recommended treatment	Same	Same		
SERB_78C	SR1_054C	SR1_054C	Reason stopped - sw - thought it was not helping	Same	Same		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
SERB_78D	SR1_054D	SR1_054D	Reason stopped - sw - problem would get better without help	Same	Same		
SERB_78E	SR1_054E	SR1_054E	Reason stopped - sw - couldn't afford to pay	Same	Same		
SERB_78F	SR1_054F	SR1_054F	Reason stopped - sw - too embarrassed to see professional	Same	Same		
SERB_78G	SR1_054G	SR1_054G	Reason stopped - sw - solve the problem without help	Same	Same		
SERB_78H	SR1_054H	SR1_054H	Reason stopped - sw - probs like transportation/ childcare	Same	Same		
SERB_78I	SR1_054I	SR1_054I	Reason stopped - sw - service / program no longer available	Same	Same		
SERB_78J	SR1_054J	SR1_054J	Reason stopped - sw - not comfortable with prof. approach	Same	Same		
	SR1_054K	SR1_054K	Reason stopped - sw - discrimination or unfair treatment	Same	Added		
SERB_78K	SR1_054L	SR1_054L	Reason stopped - sw - other reason	Same	Same		
		SR1_60AA	Talked with family member - in person	Added			
		SR1_60AB	Talked with family member - over the phone	Added			
		SR1_60AC	Talked with family member - using video	Added			
		SR1_60AD	Talked with family member - text message	Added			

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
	SR1_060	SR1_060B	Number of times consulted family member - 12 mo	Same	Added		
	SR1_062	SR1_062	Perceived level of help received - family member	Same	Added		
		SR1_70AA	Talked with friend - in person	Added			
		SR1_70AB	Talked with friend - over the phone	Added			
		SR1_70AC	Talked with friend - using video	Added			
		SR1_70AD	Talked with friend - text message	Added			
	SR1_070	SR1_070B	Number of times consulted friend - 12 mo	Same	Added		
	SR1_072	SR1_072	Perceived level of help received - friend	Same	Added		
		SR1_80AA	Talked with co-worker or supervisor - in person	Added			
		SR1_80AB	Talked with co-worker or supervisor - over the phone	Added			
		SR1_80AC	Talked with co-worker or supervisor - using video	Added			
		SR1_80AD	Talked with co-worker or supervisor - text message	Added			
	SR1_080	SR1_080B	Number of times consulted co-worker or supervisor - 12 mo	Same	Added		
	SR1_082	SR1_082	Perceived level of help received - co-work/supervisor	Same	Added		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
		SR1100AA	Talked with other people - in person	Added			
		SR1100AB	Talked with other people - over the phone	Added			
		SR1100AC	Talked with other people - using video	Added			
		SR1100AD	Talked with other people - text message	Added			
	SR1_100	SR1_100B	Number of times consulted other person - 12 mo	Same	Added		
	SR1_102	SR1_102	Perceived level of help received - other person	Same	Added		
	SR1_110	SR1_110	Received help or services from employer - 12 months	Same	Added		
	SR1_111	SR1_111	Used Internet for info/help/support - 12 m	Same	Added		
	SR1_112A	SR1_112A	Used Internet - learn about symptoms	Same	Added		
	SR1_112B	SR1_112B	Used Internet - to find out where to get help	Same	Added		
	SR1_112C	SR1_112C	Used Internet - discuss with other people	Same	Added		
	SR1_112D	SR1_112D	Used Internet - to get online therapy	Same	Added		
	SR1_112E	SR1_112E	Use Internet - other	Same	Added		
	SR1_113	SR1_113	Self-help group - help with problems - 12 months	Same	Added		
	SR1_114A	SR1_114A	Self-help group type-emotional or mental health	Same	Added		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
	SR1_114B	SR1_114B	Self-help group type - alcohol or drug use	Same	Added		
	SR1_114C	SR1_114C	Self-help group type - other	Same	Added		
SERB_A1D	SR1_115	SR1_115	Nbr of times went to meeting of self-help group - 12 months	Same	Same		
	SR1_116	SR1_116	Used telephone helpline for mental health - 12 months	Same	Added		
SERB_A2D	SR1_117	SR1_117	Number of times used telephone helpline - 12 mo	Same	Same		
SERB_A3	SR1_118	SR1_118	Needed help for emotions but didn't receive it/12 months	Same	Same		
SERB_A7	SR1_119	SR1_119	Amount spent on mental health services/products/12 months	Same	Same		
	SR1_121	SR1_121	Prejudice / discrimination - mental health	Same	Added		
	SR1_120	SR1_122	Prejudice / discrimination - personal characteristics	Same	Added		
SERBFLAP	SR1FPRU	SR1FPRU	Consultation/services used - professional - 12 m (F)	Same	Added		
	SR1FNPU	SR1FNPU	Consultation/services used - non-professional - 12 m (F)	Same	Added		
	SR1DYPRT	SR1DYPRT	Number of types - professionals consulted - 12 m (D)	Same	Added		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
	SR1DYNPT	SR1DYNPT	Number of types - non-professionals consulted - 12 m (D)	Same	Added		
	SR1DPRO1	SR1DPRO1	Hours of consultation - psychiatrist - 12 m (D)	Modified	Added	Small technical change, concept unchanged. See DV specifications.	
	SR1DPRO2	SR1DPRO2	Hours of consultation - family doctor - 12 m (D)	Modified	Added	Small technical change, concept unchanged. See DV specifications.	
	SR1DPRO3	SR1DPRO3	Hours of consultation - psychologist - 12 m (D)	Modified	Added	Small technical change, concept unchanged. See DV specifications.	
	SR1DPRO4	SR1DPRO4	Hours of consultation - nurse - 12 m (D)	Modified	Added	Small technical change, concept unchanged. See DV specifications.	
	SR1DPRO5	SR1DPRO5	Hours of consultation - social worker/counsellor - 12 m (D)	Modified	Added	Small technical change, concept unchanged. See DV specifications.	
	SR1DPROT	SR1DPROT	Hours of consultation - all professionals - 12 m (D)	Modified	Added	Small technical change, concept unchanged. See DV specifications.	
	MED_01	MED_01	Medication for emotions/mental health/alcohol/drug - 12 m	Same	Added		
	MED_02	MED_02	Num med taken for emot/mental health/alcohol/drug - 2 days	Same	Modified		Question reworded
MEDB_4	MED_05	MED_05	Insurance that covers all/part of cost of prescription	Same	Same		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
MEDB_5	MED_06	MED_06	Used other health products for mental health - 12 m	Same	Same		
	PNC_01A	PNC_01A	Received help - information	Same	Added		
	PNC_01B	PNC_01B	Received help - medication	Same	Added		
	PNC_01C	PNC_01C	Received help - counselling or therapy	Same	Added		
	PNC_01D	PNC_01D	Received help - other	Same	Added		
	PNC_01E	PNC_01E	Received help - none	Same	Added		
	PNC_02A	PNC_02A	Received - as much help as needed	Same	Added		
	PNC_02BA	PNC_02BA	Needed more help - information	Same	Added		
	PNC_02BB	PNC_02BB	Needed more help - medication	Same	Added		
	PNC_02BC	PNC_02BC	Needed more help - counselling or therapy	Same	Added		
	PNC_02BD	PNC_02BD	Needed more help - other	Same	Added		
	PN1_01A1	PNC_03AA	Why no more information / self managed	Same	Added		
	PN1_01B1	PNC_03AB	Why no more information / don't know how/where	Same	Added		
	PN1_01C1	PNC_03AC	Why no more information / too busy	Same	Added		
	PN1_01D1	PNC_03AD	Why no more information / job interfered	Same	Added		
	PN1_01E1	PNC_03AE	Why no more information / not readily available	Same	Added		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
	PN1_01F1	PNC_03AF	Why no more information / no confidence in system	Same	Added		
	PN1_01G1	PNC_03AG	Why no more information / unaffordable	Same	Added		
	PN1_01H1	PNC_03AH	Why no more information / insurance not cover	Same	Added		
	PN1_01I1	PNC_03AI	Why no more information / afraid of others opinion	Same	Added		
	PN1_01J1	PNC_03AJ	Why no more information / language problems	Same	Added		
	PN1_01K1	PNC_03AK	Why no more information / help is ongoing	Same	Added		
	PN1_01L1	PNC_03AL	Why no more information / other specify	Same	Added		
	PN1_02A1	PNC_3A1A	Why preferred to self manage / more information	Same	Added		
	PN1_01A2	PNC_03BA	Why no more medication / self managed	Same	Added		
	PN1_01B2	PNC_03BB	Why no more medication / don't know how/where	Same	Added		
	PN1_01C2	PNC_03BC	Why no more medication / too busy	Same	Added		
	PN1_01D2	PNC_03BD	Why no more medication / job interfered	Same	Added		
	PN1_01E2	PNC_03BE	Why no more medication / not readily available	Same	Added		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
	PN1_01F2	PNC_03BF	Why no more medication / no confidence in system	Same	Added		
	PN1_01G2	PNC_03BG	Why no more medication / unaffordable	Same	Added		
	PN1_01H2	PNC_03BH	Why no more medication / insurance not cover	Same	Added		
	PN1_01I2	PNC_03BI	Why no more medication / afraid of others opinion	Same	Added		
	PN1_01J2	PNC_03BJ	Why no more medication / language problems	Same	Added		
	PN1_01K2	PNC_03BK	Why no more medication / help is ongoing	Same	Added		
	PN1_01L2	PNC_03BL	Why no more medication / other specify	Same	Added		
	PN1_02A2	PNC_3B2B	Why preferred to self manage / more medication	Same	Added		
	PN1_01A3	PNC_03CA	Why no more counselling / self managed	Same	Added		
	PN1_01B3	PNC_03CB	Why no more counselling / don't know how/where	Same	Added		
	PN1_01C3	PNC_03CC	Why no more counselling / too busy	Same	Added		
	PN1_01D3	PNC_03CD	Why no more counselling/ job interfered	Same	Added		
	PN1_01E3	PNC_03CE	Why no more counselling / not readily available	Same	Added		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
	PN1_01F3	PNC_03CF	Why no more counselling / no confidence in system	Same	Added		
	PN1_01G3	PNC_03CG	Why no more counselling / unaffordable	Same	Added		
	PN1_01H3	PNC_03CH	Why no more counselling / insurance not cover	Same	Added		
	PN1_01I3	PNC_03CI	Why no more counselling / afraid of others opinion	Same	Added		
	PN1_01J3	PNC_03CJ	Why no more counselling / language problems	Same	Added		
	PN1_01K3	PNC_03CK	Why no more counselling / help is ongoing	Same	Added		
	PN1_01L3	PNC_03CL	Why no more counselling / other specify	Same	Added		
	PN1_02A3	PNC_3C3C	Why preferred to self manage / more counselling	Same	Added		
	PN1_01A4	PNC_03DA	Why no more - other / self managed	Same	Added		
	PN1_01B4	PNC_03DB	Why no more - other / don't know how/where	Same	Added		
	PN1_01C4	PNC_03DC	Why no more - other / too busy	Same	Added		
	PN1_01D4	PNC_03DD	Why no more - other / job interfered	Same	Added		
	PN1_01E4	PNC_03DE	Why no more - other / not readily available	Same	Added		
	PN1_01F4	PNC_03DF	Why no more - other / no confidence in system	Same	Added		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
	PN1_01G4	PNC_03DG	Why no more - other / unaffordable	Same	Added		
	PN1_01H4	PNC_03DH	Why no more - other / insurance not cover	Same	Added		
	PN1_01I4	PNC_03DI	Why no more - other / afraid of others opinion	Same	Added		
	PN1_01J4	PNC_03DJ	Why no more - other / language problems	Same	Added		
	PN1_01L4	PNC_03DL	Why no more - other / other specify	Same	Added		
	PN1_02A4	PNC_3D4D	Why preferred to self manage / other	Same	Added		
	PNC_04A	PNC_04A	Help needed but not received	Same	Added		
	PNC_04BA	PNC_04BA	Help needed & not received - information	Same	Added		
	PNC_04BB	PNC_04BB	Help needed & not received - medication	Same	Added		
	PNC_04BC	PNC_04BC	Help needed & not received - counselling or therapy	Same	Added		
	PN1_01A5	PNC_05BA	Why no information / self managed	Same	Added		
	PN1_01B5	PNC_05BB	Why no information / don't know how/where	Same	Added		
	PN1_01C5	PNC_05BC	Why no information / too busy	Same	Added		
	PN1_01D5	PNC_05BD	Why no information / job interfered	Same	Added		
	PN1_01E5	PNC_05BE	Why no information / not readily available	Same	Added		
	PN1_01F5	PNC_05BF	Why no information / no confidence in system	Same	Added		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
	PN1_01G5	PNC_05BG	Why no information / unaffordable	Same	Added		
	PN1_01H5	PNC_05BH	Why no information / insurance not cover	Same	Added		
	PN1_01I5	PNC_05BI	Why no information / afraid of others opinion	Same	Added		
	PN1_01J5	PNC_05BJ	Why no information / language problems	Same	Added		
	PN1_01L5	PNC_05BL	Why no information / other specify	Same	Added		
	PN1_02A5	PNC_5B2B	Why preferred to self manage / information	Same	Added		
	PN1_01A6	PNC_05CA	Why no medication / self managed	Same	Added		
	PN1_01B6	PNC_05CB	Why no medication / don't know how/where	Same	Added		
	PN1_01C6	PNC_05CC	Why no medication / too busy	Same	Added		
	PN1_01D6	PNC_05CD	Why no medication / job interfered	Same	Added		
	PN1_01E6	PNC_05CE	Why no medication / not readily available	Same	Added		
	PN1_01F6	PNC_05CF	Why no medication / no confidence in system	Same	Added		
	PN1_01G6	PNC_05CG	Why no medication / unaffordable	Same	Added		
	PN1_01H6	PNC_05CH	Why no medication / insurance not cover	Same	Added		
	PN1_01I6	PNC_05CI	Why no medication / afraid of others opinion	Same	Added		
	PN1_01J6	PNC_05CJ	Why no medication / language problems	Same	Added		
	PN1_01L6	PNC_05CL	Why no medication / other specify	Same	Added		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
	PN1_02A6	PNC_5C3C	Why preferred to self manage / medication	Same	Added		
	PN1_01A7	PNC_05DA	Why no counselling / self managed	Same	Added		
	PN1_01B7	PNC_05DB	Why no counselling / don't know how/where	Same	Added		
	PN1_01C7	PNC_05DC	Why no counselling / too busy	Same	Added		
	PN1_01D7	PNC_05DD	Why no counselling / job interfered	Same	Added		
	PN1_01E7	PNC_05DE	Why no counselling / not readily available	Same	Added		
	PN1_01F7	PNC_05DF	Why no counselling / no confidence in system	Same	Added		
	PN1_01G7	PNC_05DG	Why no counselling / unaffordable	Same	Added		
	PN1_01H7	PNC_05DH	Why no counselling / insurance not cover	Same	Added		
	PN1_01I7	PNC_05DI	Why no counselling / afraid of others opinion	Same	Added		
	PN1_01J7	PNC_05DJ	Why no counselling / language problems	Same	Added		
	PN1_01L7	PNC_05DL	Why no counselling / other specify	Same	Added		
	PN1_02A7	PNC_5D4D	Why preferred to self manage / counselling	Same	Added		
	PNCFH12	PNCFH12	Flag: Help received - past 12 months (F)	Same	Added		
	PNCDHCT	PNCDHCT	Number of types of help received - past 12 months - (D)	Same	Added		
	PNCDPNI	PNCDPNI	Perceived need for information - past 12 months (D)	Same	Added		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
	PNCDPNM	PNCDPNM	Perceived need for medication - past 12 months (D)	Same	Added		
	PNCDPNC	PNCDPNC	Perceived need for counselling - past 12 months (D)	Same	Added		
	PNCDPNO	PNCDPNO	Perceived need for other help - past 12 months (D)	Same	Added		
	PNCDNEED	PNCDNEED	Overall Perceived Need in Past 12 Months - (D)	Same	Added		
	MHE_05A	MHE_05A	Received mental health treatment - life	Same	Added		
	MHE_05B	MHE_05B	Received mental health treatment - 12 m	Same	Added		
	MHE_06	MHE_06	Perceived prejudice/discrimination due to mental health -12m	Same	Added		
	MHE_06A	MHE_06A	Impact - mental health prej/discrimin - family relationships	Same	Added		
	MHE_06B	MHE_06B	Impact - mental health prej/discrim - romantic life	Same	Added		
	MHE_06C	MHE_06C	Impact - mental health prej/discrim - work/school	Same	Added		
	MHE_06D	MHE_06D	Impact - mental health prej/discrim - financial situation	Same	Added		
	MHE_06E	MHE_06E	Impact - mental health prej/discrim - housing	Same	Added		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
	MHE_06F	MHE_06F	Impact - mental health prej/discrim - health care/physical	Same	Added		
	FMI_01A	FMI_01A	Family member - mental health/alcohol/drug problems	Same	Added		
	FMI_02	FMI_02	Life affected by family member's problem	Same	Added		
	SPS_01	SPS_01	Has people to depend on	Same	Added		
	SPS_02	SPS_02	Has people who enjoy the same activities	Same	Added		
	SPS_03	SPS_03	Has close relationships	Same	Added		
	SPS_04	SPS_04	Has someone to talk to about important decisions	Same	Added		
	SPS_05	SPS_05	Has relationships that recognize competence/skill	Same	Added		
	SPS_06	SPS_06	Has trustworthy person for advice	Same	Added		
	SPS_07	SPS_07	Feel part of a group who shared attitudes/beliefs	Same	Added		
	SPS_08	SPS_08	Has strong emotional bond	Same	Added		
	SPS_09	SPS_09	Has people who admire talents/abilities	Same	Added		
	SPS_10	SPS_10	Has people to count on in emergency	Same	Added		
	SPSDCON	SPSDCON	Social Provisions Scale - overall score - (D)	Same	Added		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
	SPSDATT	SPSDATT	Social Provisions Scale - attachment - (D)	Same	Added		
	SPSDGUI	SPSDGUI	Social Provisions Scale - guidance - (D)	Same	Added		
	SPSDALL	SPSDALL	Social Provisions Scale - reliable alliance - (D)	Same	Added		
	SPSDINT	SPSDINT	Social Provisions Scale - social integration - (D)	Same	Added		
	SPSDWOR	SPSDWOR	Social Provisions Scale - reassurance of worth (D)	Same	Added		
	NSI_01	NSI_01	Negative social interactions - detrimental to well-being	Same	Added		
	NSI_02	NSI_02	Negative social interactions - made too many demands	Same	Added		
	NSI_03	NSI_03	Negative social interactions - critical of behaviour	Same	Added		
	NSI_04	NSI_04	Negative social interactions - thoughtless/inconsiderate	Same	Added		
	NSI_05	NSI_05	Negative social interactions - angry/upset	Same	Added		
	NSIDSC	NSIDSC	Negative social interactions - scale - (D)	Same	Added		
	CEX_01	CEX_01	Num of times - saw/heard parent hit other adult in your home	Same	Added		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
	CEX_02	CEX_02	Num of times - slapped/hit/spanked	Same	Added		
	CEX_03	CEX_03	Num of times - pushed/grabbed/shoved/threw things at you	Same	Added		
	CEX_04	CEX_04	Num of times - physically attacked	Same	Added		
	CEX_05	CEX_05	Num of times - forced unwanted sexual activity	Same	Added		
	CEX_06	CEX_06	Num of times - sexual touching	Same	Added		
	CEX_07	CEX_07	Saw/talked w child protection organization	Same	Added		
	CEXDNUM	CEXDNUM	Number of types of childhood maltreatment experienced (D)	Same	Added		
SPVB_1	SPT_01	SPI_01	Spiritual values - play important role in life	Same	Modified		Question reworded
LBFB_01	LBS_01	LMAM_01	Worked at job or business last week	Same	Same		
LBFB_02	LBS_02	LMAM_02	Absent from job or business last week	Same	Same		
		LMAM_03	Absent from job / business - reason - last week	Added			
	LBSCSOC	LBSCSOC	National Occupational Classification (NOC) 2011	Same	Added		
LBSBDWSS	LBSDWSS	LBSDWSS	Working status last week - (D)	Same	Modified		Conditions changed in DV specifications
	LBSDOCG	LBSDOCG	Occupation group - (D)	Same	Added		

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
WSTB_401	WST_401	WST_401	Work stress - learn new things	Same	Same		
WSTB_402	WST_402	WST_402	Work stress - high level of skill	Same	Same		
WSTB_403	WST_403	WST_403	Work stress - freedom to decide	Same	Same		
WSTB_404	WST_404	WST_404	Work stress - repetitive tasks	Same	Same		
WSTB_405	WST_405	WST_405	Work stress - job hectic	Same	Same		
WSTB_406	WST_406	WST_406	Work stress - free from conflicting demands	Same	Same		
WSTB_407	WST_407	WST_407	Work stress - good job security	Same	Same		
WSTB_408	WST_408	WST_408	Work stress - a lot of physical effort	Same	Same		
WSTB_409	WST_409	WST_409	Work stress - had own job input	Same	Same		
WSTB_410	WST_410	WST_410	Work stress - hostility or conflict with others	Same	Modified		"Not applicable" response category was added
WSTB_411	WST_411	WST_411	Work stress - supervisor helpful	Same	Modified		"Not applicable" response category was added
WSTB_412	WST_412	WST_412	Work stress - co-workers helpful	Same	Modified		"Not applicable" response category was added
WSTB_413	WST_413	WST_413	Work stress - job satisfaction	Same	Same		
WSTBDSKI	WSTDSKI	WSTDSKI	Work stress - decision latitude/discretion - (D)	Modified	Same	Conditions changed in DV specifications	
WSTBDAUT	WSTDAUT	WSTDAUT	Work stress - decision latitude / authority - (D)	Modified	Same	Conditions changed in DV specifications	

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
WSTBDPSY	WSTDPSY	WSTDPSY	Work stress - psychological demands - (D)	Same	Same		
WSTBDJIN	WSTDJIN	WSTDJIN	Work stress - job insecurity - (D)	Same	Same		
WSTBDPHY	WSTDPHY	WSTDPHY	Work stress - physical exertion - (D)	Same	Same		
WSTBDSOC	WSTDSOC	WSTDSOC	Work stress - social support - (D)	Modified	Same	Conditions changed in DV specifications	"Not applicable" response category was added to input variables
	WSTDJST	WSTDJST	Work stress - job strain - (D)	Modified	Added	Conditions changed in DV specifications	
		SDC_027	Canadian military service	Added			
	SDC_7AA	SDC_037	Sexual orientation	Modified	Added	Question wording updated to meet new Harmonized Content standards	
		SORSI	Sexual Identity	Added			
		SORLGBTS	LGBTQ2+ Status	Added			
		IMM_01	Place of birth	Added			
		IMM_01A1	Birth province or territory	Added			
		IMM_01A2	Birth country	Added			
SDCBCCB	SDCCCB10	SDCCCOB	Country of birth - (C)	Same	Modified		Starting with the 2011 CCHS - Annual data, this variable is an update of SDCCCB. The country code is based on the Standard Classification of Countries and Areas of Interest (SCCAI) of 2010. The SCCAI is Statistics Canada's official classification

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
							of countries and areas.
SDCBGCB	SDCGCB10	SDCDVCB	Country of birth - grouped (G)	Same	Modified		Derived variable conditions changed
	SDC_3	IMM_02	Year of arrival	Added			
		IMM_03	Immigrant status	Added			
SDCB_3	SDC_3	IMM_04	Year of immigration	Same	Same		
SDCBDAIM	SDCDAIM	SDCDAIM	Age at time of immigration - (D)	Same	Same		
SDCBFIMM	SDCFIMM	SDCFIMM	Immigrant - (F)	Same	Same		
	SDC_41	ABM_01A	Aboriginal - First Nations/ Métis/ Inuit	Same	Added		
	SDC_42A	ABM_01B	Aboriginal - First Nations	Same	Added		
	SDC_42B	ABM_01C	Aboriginal - Métis	Same	Added		
	SDC_42C	ABM_01D	Aboriginal - Inuit	Same	Added		
	SDCDABT	SDCDABT	Aboriginal identity - (D)	Modified	Added	Derived variable conditions changed, concept unchanged. See DV specifications.	
SDCB_7A	SDC_43A	PG_05A	Cultural/racial origin - White	Same	Modified		Question reworded
SDCB_7B	SDC_43C	PG_05B	Cultural/racial origin - South Asian	Same	Modified		Question reworded
SDCB_7B	SDC_43B	PG_05C	Cultural/racial origin - Chinese	Same	Modified		Question reworded
SDCB_7D	SDC_43D	PG_05D	Cultural/racial origin - Black	Same	Modified		Question reworded
SDCB_7E	SDC_43E	PG_05E	Cultural/racial origin - Filipino	Same	Modified		Question reworded
SDCB_7H	SDC_43H	PG_05F	Cultural/racial origin - Arab	Same	Modified		Question reworded
SDCB_7F	SDC_43F	PG_05G	Cultural/racial origin - Latin American	Same	Modified		Question reworded
SDCB_7G	SDC_43G	PG_05H	Cultural/racial origin - Southeast Asian	Same	Modified		Question reworded

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
SDCB_7I	SDC_43I	PG_05I	Cultural/racial origin - West Asian	Same	Modified		Question reworded
SDCB_7K	SDC_43K	PG_05J	Cultural/racial origin - Korean	Same	Modified		Question reworded
SDCB_7J	SDC_43J	PG_05K	Cultural/racial origin - Japanese	Same	Modified		Question reworded
		VMRNIE	Belonging racial / cultural groups - visible minority n.i.e.	Added			
		VMROTHER	Belonging racial / cultural groups - not a visible minority	Added			
SDCBDRAC	SDCDCGT	SDCDVVM	Visible minority group of respondent - (D)	Modified	Modified		Derived variable conditions changed
SDCBDRES	SDCDRES	SDCDRES	Length of time in Canada since immigration - (D)		Same		
		SDCDCFL	Language first learned at home and still understood - Mother tongue				
		SDCDVFLA	Visible minority	Added			
EDUB_4	EDU1_4	EDU_05	Highest degree/ certificate/ diploma obtained	Same	Same		
		ADM_035	Permission to share data	Added	Modified		Conditions changed in DV specifications
		ADM_037	Permission to share linked data	Added			
SAMDSHR	SAMDSHR	SAMDSHR	Permission to share data - (D)	Same	Same		
SAMDLNK	SAMDLNK	SAMDLNK	Permission to link data - (D)	Same	Same		
		INCP2019	Personal income from all sources for 2019	Added			
		INCP2020	Personal income from all sources for 2020	Added			

Variables 2002	Variables 2012	Variables 2022	Concept	Status 2022	Status 2012	Change Made 2022	Change Made 2012
		INCH2020	Household income from all sources for 2020	Added			
		INCDVP19	Personal income from all sources for 2019 - Grouped	Added			
		INCDVP20	Personal income from all sources for 2020 - Grouped	Added			
		INCDVHH	Household income from all sources for 2020 - Grouped	Added			
		DABTIPPE	Neighbourhood income decile before tax (in CMA/CA)	Added			
		DNBTIPPE	Neighbourhood income decile before tax (national)	Added			
WTSB_M	WTS_M	WTS_M	Weights - Master	Same	Same		

Appendix B:

WHO-CIDI Mental and Substance Disorders – Primary variables for analysis and comparable 2022, 2012, 2002 variables

This list of the primary disorder variables for analysis summarizes the variables associated with the final algorithms. See the Derived Variable (DV) Specifications for more detailed description of these variables and all the component variables. Because the suite of disorders measured in 2022 differs from those measured in 2012 and 2002, the summary variables are not comparable across surveys. Variables from the 2012 CCHS – Mental Health and 2002 CCHS – Mental Health and Well-being are listed only if they are comparable to 2022.

Concept	Variables 2022	Variables 2012	Variables 2002
Any selected disorder (mental or substance) - life (F)	MHPFL		
Any selected disorder (mental/substance) - 12 m - (F)	MHPFY		
Any selected disorder (mental/substance) - Interfere -12 m (F)	MHPFINT		
Any mood disorder - life - (F)	MHPFLM		
Any mood disorder - 12 m - (F)	MHPFYM		
Major Depressive Episode - Algorithm - life - (D)	DEPDDPS	DEPDDPS	DEPBDDPS
Major Depressive Episode - Algorithm - 12 m - (D)	DEPDDY	DEPDDY	DEPBDDY
Major depressive episode - Most recent episode - age - (D)	DEPDREC	DEPDREC	DEPBDREC
Major depressive episode - persistence - years - (D)	DEPDPER	DEPDPER	DEPBDPER
Flag: Depression - Interference - 12 m - (F)	DEPFINT	DEPFINT	DEPBFINT
Depression - Interference - Mean - 12 m - (D)	DEPDINT	DEPDINT	DEPBDINT
Flag: Suicide - thought - life - (F)	DEPFSLT	DEPFSLT	DEPBFSLT
Flag: Suicide - plan - life - (F)	DEPFSLP	DEPFSLP	
Flag: Suicide - attempt - life - (F)	DEPFSLA	DEPFSLA	DEPBFSLA
Flag: Suicide - thought - 12 m - (F)	DEPFSYT	DEPFSYT	DEPBFSYT, DEPBISYT (imputation flag)
Flag: Suicide - plan - 12 m - (F)	DEPFSYP	DEPFSYP	
Flag: Suicide - attempt - 12 m - (F)	DEPFSYA	DEPFSYA	DEPBFSYA, DEPBISYA (imputation flag)
Bipolar Disorder - Algorithm - life - (D)	BIPDL	BIPDL	
Bipolar Disorder - Algorithm - 12 m - (D)	BIPDY	BIPDY	
Bipolar Disorder - Interference - Mean - (D)	BIPDINT	BIPDINT	
Flag: Bipolar Disorder - Interference - (F)	BIPFINT	BIPFINT	
Bipolar I - Algorithm - life - (D)	BIPD1	BIPD1	
Bipolar I - Algorithm - 12 m - (D)	BIPD1Y	BIPD1Y	
Flag: Bipolar I - Interference - (F)	BIPFINT1	BIPFINT1	
Bipolar I - Interference - Mean - (D)	BIPDINT1	BIPDINT1	
Bipolar II - Algorithm - life - (D)	BIPD2	BIPD2	
Bipolar II - Algorithm - 12 m - (D)	BIPD2Y	BIPD2Y	
Flag: Bipolar II - Interference - (F)	BIPFINT2	BIPFINT2	
Bipolar II - Interference - Mean - (D)	BIPDINT2	BIPDINT2	

Concept	Variables 2022	Variables 2012	Variables 2002
Mania - 2002 Algorithm - life - (D)	MIADEPS	MIADEPS	MIABDEPS
Mania - 2002 Algorithm - 12 m - (D)	MIADEY	MIADEY	MIABDEY
Mania - Most recent episode - age (D)	MIADREC	MIADREC	MIABDREC
Mania - Persistence - years - (D)	MIADPER	MIADPER	MIABDPER
Flag: Mania - Interference - 12 m - (F)	MIAFINT	MIAFINT	MIABFINT
Mania - Interference - Mean - 12 m - (D)	MIADINT	MIADINT	MIABDINT
Generalized anxiety disorder - algorithm - life - (D)	GADDGDS	GADDGDS	
Generalized anxiety disorder - algorithm - 12 m - (D)	GADDDY	GADDDY	
Gen anx dis - Most recent episode - age - (D)	GADDREC	GADDREC	
Gen anx dis - Longest episode - duration - (D)	GADD05Y	GADD05Y	
Generalized anxiety disorder - Interference - 12 m - (F)	GADFINT	GADFINT	
Generalized anxiety disorder - Interference - Mean -12 m (D)	GADDINT	GADDINT	
Social Phobia - Algorithm - life	SOPD02		SOPBBDSP
Social Phobia - Algorithm - 12 m	SOPDPY		SOPBDPY
Social Phobia - Recency Age	SOPDREC		SOPBDREC
Social Phobia Interference - Mean	SOPDINT		SOPBDINT
Flag for Social Phobia Interference	SOPFINT		SOPBFINT
Any substance use disorder (alcohol/drug) - life - (F)	MHPFLSA		
Any substance use disorder (alcohol/drug) - 12m - (F)	MHPFYSA		
Alcohol abuse or dependence - Algorithm - life - (D)	AUDDL	AUDDL	
Alcohol abuse or dependence - Algorithm - 12 m - (D)	AUDDY	AUDDY	
Alcohol abuse - Algorithm - life - (D)	AUDDLA	AUDDLA	
Alcohol abuse - Algorithm - 12 m - (D)	AUDDYA	AUDDYA	
Alcohol dependence - Algorithm - life - (D)	AUDDLD	AUDDLD	
Alcohol dependence - Algorithm - 12 m - (D)	AUDDYD	AUDDYD	
Flag: Alcohol - Interference - 12 m - (F)	AUDFINT	AUDFINT	
Alcohol - Interference - Mean - 12 m - (D)	AUDDINT	AUDDINT	
Cannabis abuse or dependence - life - (D)	SUDDLC	SUDDLC	
Cannabis abuse or dependence - 12 m - (D)	SUDDYC	SUDDYC	
Cannabis abuse - Algorithm - life - (D)	SUDDLCA	SUDDLCA	
Cannabis abuse - Algorithm - 12 m - (D)	SUDDYCA	SUDDYCA	
Cannabis dependence - Algorithm - life - (D)	SUDDLCD	SUDDLCD	
Cannabis dependence - Algorithm - 12 m - (D)	SUDDYCD	SUDDYCD	
Drug abuse or dependence (excluding cannabis) - life - (D)	SUDDLO	SUDDLO	
Drug abuse or dependence (excluding cannabis) - 12 m - (D)	SUDDYO	SUDDYO	
Drug abuse (excluding cannabis) - Algorithm - life - (D)	SUDDLOA	SUDDLOA	
Drug abuse (excluding cannabis) - Algorithm - 12 m - (D)	SUDDYOA	SUDDYOA	
Drug dependence (excluding cannabis) - Algorithm - life - (D)	SUDDLOD	SUDDLOD	
Drug dependence (excluding cannabis) - Algorithm - 12 m - (D)	SUDDYOD	SUDDYOD	
Substance use - Interference - 12 m - (F)	SUDFINT	SUDFINT	
Substance use - Interference - Mean - 12 m (D)	SUDDINT	SUDDINT	